#1 · Musculoskeletal Physiotherapy

Which findings after an acute ankle inversion injury mean the Ottawa ankle rules call for an X-ray?

Answer

Bone tenderness at the posterior edge or tip of either malleolus, or at the base of the fifth metatarsal or navicular, or inability to take four steps. These screen for fracture and reduce unnecessary imaging.

#2 · Musculoskeletal Physiotherapy

A patient has pain above the ankle joint after a dorsiflexion and external rotation injury. Which injury should you suspect and test for?

Answer

Syndesmosis (high ankle) sprain. Use the squeeze test and external rotation stress test. It heals more slowly than a lateral sprain and may need protection, so do not progress loading as for a standard sprain.

#3 · Musculoskeletal Physiotherapy

What is the preferred approach to loading in the early phase of a stable grade 2 lateral ankle sprain?

Answer

Early protected weight bearing and graded movement within pain limits, with support such as a brace or tape if needed. Prolonged immobilisation delays recovery, while early controlled loading improves function and return to activity.

#4 · Musculoskeletal Physiotherapy

Why is balance and proprioceptive training included in rehabilitation after a lateral ankle sprain?

Answer

It restores neuromuscular control of the ankle and reduces the risk of recurrent sprains, which is the commonest long-term problem. Progress from double leg to single leg, then unstable surfaces, then sport-specific tasks.

#5 · Musculoskeletal Physiotherapy

You applied the Ottawa ankle rules and referred a patient for imaging. What should your documentation record?

Answer

Each rule finding (positive or negative), the weight-bearing result, your clinical reasoning, the referral made and the advice given. This supports continuity of care and shows your decision was evidence-based and safe.

#6 · Musculoskeletal Physiotherapy

What is the typical mechanism and presentation of an acute hamstring strain in a sprinter?

Answer

Sudden sharp posterior thigh pain during high-speed running, usually in late swing phase when the muscle lengthens under load. Expect pain with resisted knee flexion, stretch and palpation, and sometimes bruising a few days later.

#7 · Musculoskeletal Physiotherapy

Which presentation after a hamstring injury suggests proximal avulsion at the ischial tuberosity and needs urgent medical referral?

Answer

Severe pain after a forced hip flexion with knee extension, a palpable gap or bunching, extensive bruising and marked weakness. Avulsion with significant displacement may need surgical review, so avoid aggressive stretching and refer.

#8 · Musculoskeletal Physiotherapy

Which exercise has good evidence for reducing hamstring strain injury risk in sport?

Answer

The Nordic hamstring exercise, an eccentric knee flexion exercise, when done regularly as part of a warm-up or training programme. Introduce volume gradually to avoid soreness, and combine it with sprint exposure for best results.

#9 · Musculoskeletal Physiotherapy

What criteria should an athlete meet before returning to full competitive sprinting after a hamstring strain?

Answer

Pain-free full range, near-symmetrical strength on resisted testing, and progressive pain-free sprint and sport drills at increasing speed. Using criteria rather than time alone lowers the high risk of re-injury.

#10 · Musculoskeletal Physiotherapy

A coach asks you for details of an athlete's hamstring diagnosis and rehabilitation plan. What must you do first?

Answer

Obtain the athlete's consent before sharing health information, and share only what they agree to. Privacy obligations apply to identifiable health information, even in team settings, and the athlete decides what the coach is told.

#11 · Musculoskeletal Physiotherapy

How do you test the medial collateral ligament of the knee, and in what positions?

Answer

Apply a valgus stress at about 30 degrees of knee flexion, and also in full extension. Compare with the other side, noting pain, end-feel and joint opening, which indicates the grade of sprain.

#12 · Musculoskeletal Physiotherapy

Laxity to valgus stress with the knee in full extension suggests what after a medial knee injury?

Answer

A more severe injury involving the posteromedial capsule and possibly the cruciate ligaments, not an isolated MCL sprain. This needs further assessment and medical or imaging referral, since combined injuries change management.

#13 · Musculoskeletal Physiotherapy

Which other structures are commonly injured with an MCL sprain after a valgus contact injury?

Answer

The ACL and the medial meniscus (the terrible triad includes all three). Always test the cruciates and menisci and ask about swelling, locking and instability so associated injuries are not missed.

#14 · Musculoskeletal Physiotherapy

What is the usual management approach for a grade 1 to 2 MCL sprain?

Answer

Most heal without surgery. Use relative protection such as a hinged brace if needed, early range of movement, progressive strengthening and gradual return to function. Avoid valgus loading while the ligament is healing.

#15 · Musculoskeletal Physiotherapy

You delegate supervised strengthening for a patient with an MCL sprain to an allied health assistant. What must you retain?

Answer

Responsibility for assessment, the exercise prescription, progression and monitoring. The assistant carries out a clear written plan, you check they are competent, and you review the patient regularly and adjust the plan.

#16 · Musculoskeletal Physiotherapy

Which findings support rotator cuff tendinopathy rather than a frozen shoulder?

Answer

Painful arc on elevation, pain and weakness on resisted external rotation or abduction, and passive range that is relatively preserved. In frozen shoulder, passive and active motion are both markedly limited, especially external rotation.

#17 · Musculoskeletal Physiotherapy

Which signs suggest a full-thickness rotator cuff tear rather than tendinopathy?

Answer

Marked weakness, a positive drop arm test or external rotation lag sign, and difficulty holding the arm against gravity after trauma. Refer for imaging and orthopaedic opinion, because early repair may be considered in younger patients.

#18 · Musculoskeletal Physiotherapy

What is the main physiotherapy approach for rotator cuff tendinopathy?

Answer

Progressive loading of the rotator cuff and scapular muscles, with education on load management and activity modification. Exercise is first-line treatment and should be progressed as pain and capacity allow.

#19 · Musculoskeletal Physiotherapy

What does the Shoulder Pain and Disability Index (SPADI) measure and how does it guide treatment?

Answer

It is a patient-reported measure of shoulder pain and disability in daily activities. Use it at baseline and review to track change and set functional goals, rather than relying on pain alone.

#20 · Musculoskeletal Physiotherapy

A patient with shoulder pain reports unremitting night pain, weight loss and a history of cancer. What is your action?

Answer

Treat these as red flags for serious pathology such as metastasis. Stop usual treatment and refer promptly to the GP or specialist, documenting the findings and your reasoning.

#21 · Musculoskeletal Physiotherapy

Which tests help identify lateral epicondylalgia and what do they reproduce?

Answer

Palpation of the lateral epicondyle, resisted wrist extension (Cozen's test), resisted middle finger extension and pain on gripping. A positive result reproduces the lateral elbow pain over the common extensor origin.

#22 · Musculoskeletal Physiotherapy

What is the evidence-based first approach to physiotherapy management of lateral epicondylalgia?

Answer

Education, activity modification and progressive loading of the wrist extensors, such as isometric then isotonic or eccentric exercise. Grip and shoulder strengthening may also help, as the condition is often slow to settle.

#23 · Musculoskeletal Physiotherapy

What does the evidence say about corticosteroid injection for lateral epicondylalgia compared with exercise?

Answer

Injections may give short-term relief but are associated with worse outcomes and more recurrence at longer follow-up. Explain this so the patient can make an informed choice, and favour education and loading.

#24 · Musculoskeletal Physiotherapy

What does the Patient-Rated Tennis Elbow Evaluation (PRTEE) measure and how is it used?

Answer

It measures patient-reported elbow pain and function in specific and usual activities. Record it at baseline and at review to track progress and guide goals, alongside pain-free grip strength and return to work tasks.

#25 · Musculoskeletal Physiotherapy

A worker with lateral epicondylalgia has a compensable claim. What is your role in return to work planning?

Answer

Assess work capacity, recommend suitable duties and modifications, and communicate with the insurer and employer as the scheme requires, with the patient's consent. Document functional restrictions and progress to support graded return to work.

#26 · Musculoskeletal Physiotherapy

A runner has morning stiffness and pain 2 to 6 cm above the Achilles insertion. What does this location suggest, and how does it guide management?

Answer

This suggests mid-portion Achilles tendinopathy. It responds well to progressive loading (isometric, then isotonic, then energy-storage work). Insertional pain near the calcaneus needs modified loading that avoids deep dorsiflexion compression.

#27 · Musculoskeletal Physiotherapy

What is the purpose of the Royal London Hospital test and the arc sign in suspected Achilles tendinopathy?

Answer

They help localise a tendon source of pain. The arc sign: swelling moves with ankle motion when the tendon is the pain source. The Royal London test: tenderness lessens in dorsiflexion, which points to tendon rather than paratenon.

#28 · Musculoskeletal Physiotherapy

A patient reports a sudden pop and weakness in the calf, and cannot do a single-leg heel raise. What must you rule out and what do you do?

Answer

Rule out Achilles tendon rupture. Check for a palpable gap and a positive Simmonds (calf squeeze) test. A suspected rupture needs urgent medical or orthopaedic referral and protection of the tendon, not loading exercises.

#29 · Musculoskeletal Physiotherapy

Why is complete rest usually a poor plan for Achilles tendinopathy, and what should you teach about load instead?

Answer

Rest reduces tendon capacity and the pain returns on resumption. Teach load management: keep activity within a tolerable pain response, vary the load, and build capacity gradually with a graded programme.

#30 · Musculoskeletal Physiotherapy

A patient with Achilles tendinopathy speaks limited English and relies on a family member to interpret. What is the better practice?

Answer

Book a qualified interpreter (such as TIS National or a health service interpreter). Family members may filter or alter information, breach confidentiality, and compromise informed consent. Document that an interpreter was used.

#31 · Musculoskeletal Physiotherapy

Which classic features distinguish patellofemoral pain from other anterior knee conditions on history?

Answer

Diffuse anterior knee pain around or behind the patella, aggravated by loaded knee flexion such as stairs, squatting, prolonged sitting and running. No locking or giving way from instability, and no significant effusion.

#32 · Musculoskeletal Physiotherapy

What does the single-leg squat or step-down test tell you in patellofemoral pain, and what is it not?

Answer

It reproduces symptoms under load and shows how the patient controls the trunk, pelvis and knee. It is a functional observation to guide exercise, not a diagnostic test, and quality of movement must be judged with caution.

#33 · Musculoskeletal Physiotherapy

What is a core component of first-line management for patellofemoral pain, according to current consensus?

Answer

Exercise therapy targeting the hip and knee muscles (for example quadriceps and gluteal strengthening), combined with education and load management. Taping, foot orthoses or bracing may be added as short-term adjuncts.

#34 · Musculoskeletal Physiotherapy

A young patient with knee pain after a fall has a large effusion, locking and inability to straight-leg raise. What does this change?

Answer

These features suggest an intra-articular injury or extensor mechanism problem, not simple patellofemoral pain. Arrange medical review and imaging before progressing exercise, and apply the Ottawa knee rules to guide the need for radiography.

#35 · Musculoskeletal Physiotherapy

When can an allied health assistant deliver a patellofemoral exercise programme, and who stays accountable?

Answer

The physiotherapist assesses, writes the plan and delegates specific, appropriate tasks to a competent assistant with adequate supervision. The physiotherapist remains accountable for the patient's care and reviews progress.

#36 · Musculoskeletal Physiotherapy

What typical history and examination findings point to greater trochanteric pain syndrome?

Answer

Lateral hip pain, often worse lying on the affected side, with stairs or standing on one leg. Palpation over the greater trochanter reproduces pain, and single-leg stance for about 30 seconds often provokes symptoms.

#37 · Musculoskeletal Physiotherapy

Why should you avoid prolonged hip adduction postures (crossing legs, sleeping on the painful side without support) in this condition?

Answer

Adduction compresses the gluteal tendons against the greater trochanter and bursa, increasing tendon load and pain. Advise a pillow between the knees and avoiding standing with the hips hanging into adduction.

#38 · Musculoskeletal Physiotherapy

What is the preferred exercise approach for greater trochanteric pain syndrome in the early stage?

Answer

Education plus progressive gluteal tendon loading, starting with isometric work and avoiding stretching into adduction. Increase load gradually as pain settles, because compression and tension both aggravate irritable tendons.

#39 · Musculoskeletal Physiotherapy

A patient has lateral hip pain but also groin pain, reduced hip internal rotation and night pain. What should you consider?

Answer

Hip joint pathology such as osteoarthritis may be present or coexisting, and lateral pain alone should not be assumed tendon related. Examine the hip joint and consider medical review or imaging if findings are inconsistent.

#40 · Musculoskeletal Physiotherapy

What should you document and discuss before referring a patient with GTPS for a corticosteroid injection opinion?

Answer

Document that conservative loading has been tried and the response. Explain that injections may give short-term relief but benefit often fades, and refer to the GP or specialist, who decides prescribing and procedures.

#41 · Musculoskeletal Physiotherapy

What features define non-specific mechanical low back pain without red flags?

Answer

Pain varies with posture and movement, has no clear structural cause, and has no neurological deficit or serious pathology signs. Most cases improve over weeks, so reassurance and staying active are central.

#42 · Musculoskeletal Physiotherapy

Name four red flags in low back pain that need urgent medical attention.

Answer

Saddle anaesthesia, new bladder or bowel dysfunction, progressive bilateral leg weakness (suggesting cauda equina), and suspected fracture, malignancy or infection. Escalate to emergency or urgent medical care the same day.

#43 · Musculoskeletal Physiotherapy

Which psychosocial 'yellow flags' predict persistent disability in low back pain, and how do they change your approach?

Answer

Fear of movement, catastrophising, low mood, poor work satisfaction and expectation of poor recovery. Address them with education, graded activity and goal setting, and consider screening tools such as the STarT Back tool to guide care.

#44 · Musculoskeletal Physiotherapy

What do current guidelines recommend for imaging in acute low back pain without red flags?

Answer

Routine imaging is not recommended. Findings often match age-related change and can increase fear and unnecessary treatment. Reserve imaging for red flags or when results would change management.

#45 · Musculoskeletal Physiotherapy

Which outcome measure captures disability from low back pain and guides goal setting?

Answer

The Oswestry Disability Index or the Roland-Morris Disability Questionnaire measure how back pain affects daily activities. Repeat them to track change and tie goals to patient-chosen functional tasks.

#46 · Musculoskeletal Physiotherapy

A patient has leg pain below the knee, a positive slump test and reduced ankle reflex. What does this suggest, and which root is likely?

Answer

Lumbar radiculopathy with nerve root involvement. A reduced Achilles reflex typically points to S1. Map dermatome sensation and myotome strength (plantarflexion for S1) to confirm the level and track change.

#47 · Musculoskeletal Physiotherapy

What is the purpose of the slump test, and what counts as a positive result?

Answer

It tensions the neural system to test for neural mechanosensitivity. It is positive when it reproduces the patient's familiar leg symptoms and these change with cervical or ankle movement (structural differentiation).

#48 · Musculoskeletal Physiotherapy

Radiculopathy patient develops progressive foot drop over a week. What do you do?

Answer

Progressive motor loss is a sign of significant nerve compression. Refer urgently to the GP or a spinal specialist for imaging and surgical opinion, document the myotome grades, and avoid techniques that might aggravate the deficit.

#49 · Musculoskeletal Physiotherapy

What is the usual natural history of lumbar radiculopathy and what does it mean for early management?

Answer

Most cases improve with time, often within weeks to months. Provide education, reassurance, pain-guided activity and neural-friendly exercise, and monitor neurology regularly while arranging referral if symptoms fail to improve.

#50 · Musculoskeletal Physiotherapy

A patient with radiculopathy is on workers compensation. What documentation and funding practice applies?

Answer

Follow the insurer's claim requirements: confirm the claim and approval before treatment, document the injury mechanism, progress, work capacity and goals, and report to the case manager as required, while keeping the patient's information confidential.

#51 · Musculoskeletal Physiotherapy

A patient with lumbar spinal stenosis walks 100 metres before leg symptoms start. What feature points to neurogenic rather than vascular claudication?

Answer

Relief on sitting or leaning forward, and symptoms provoked by standing as well as walking, suggest neurogenic claudication. Lumbar flexion widens the canal. Vascular claudication eases with simple standing rest and is not posture dependent.

#52 · Musculoskeletal Physiotherapy

Why do patients with neurogenic claudication often tolerate stationary cycling better than walking?

Answer

Cycling keeps the lumbar spine flexed, which enlarges the spinal canal and foramina and reduces nerve root compression. Walking encourages extension, which narrows the canal. This makes cycling a useful aerobic option for graded activity.

#53 · Musculoskeletal Physiotherapy

Which new symptoms in a patient with known spinal stenosis require urgent medical referral?

Answer

Saddle numbness, new bladder or bowel dysfunction, sexual dysfunction or rapidly progressing bilateral leg weakness suggest cauda equina compromise. This is a surgical emergency, so send the patient to an emergency department the same day.

#54 · Musculoskeletal Physiotherapy

What outcome measure suits tracking walking tolerance in a patient with neurogenic claudication?

Answer

Use a timed or distance walking test, such as the self-paced walking test on a treadmill, alongside a patient-reported measure of function. Walking distance before symptoms start is the key functional limit and guides goals and progression.

#55 · Musculoskeletal Physiotherapy

A patient with lumbar stenosis asks to see you again after being told surgery may be needed. How do you handle consent and referral?

Answer

Explain the options and evidence, respect the patient's choice, and refer to the GP or a spinal specialist if conservative care fails or neurological signs progress. Document the discussion and the patient's decision, as consent is ongoing.

#56 · Musculoskeletal Physiotherapy

What is the usual initial priority when managing acute whiplash associated disorder?

Answer

Reassure the patient, explain that recovery is expected, and encourage early return to normal activity and gentle active neck movement. Rest and prolonged collar use delay recovery. Screening for serious injury comes first.

#57 · Musculoskeletal Physiotherapy

Which tool helps decide whether a neck injury after a collision needs imaging?

Answer

The Canadian C-spine rule or NEXUS criteria help identify patients at risk of cervical fracture. Factors such as age 65 or over, dangerous mechanism, paraesthesia, or inability to rotate the neck 45 degrees each way raise the need for imaging.

#58 · Musculoskeletal Physiotherapy

What yellow flags predict poor recovery after whiplash?

Answer

High initial pain intensity, fear of movement, catastrophising, low expectation of recovery, psychological distress and ongoing compensation disputes predict slower recovery. Identify them early and address them with education, graded activity and appropriate referral.

#59 · Musculoskeletal Physiotherapy

Which outcome measure is commonly used for neck pain related disability in whiplash?

Answer

The Neck Disability Index measures how neck pain affects daily activities, giving a baseline and a way to track change. Repeat it at set intervals to show progress and to support goal setting and discharge planning.

#60 · Musculoskeletal Physiotherapy

A whiplash patient has a compulsory third party (CTP) claim. What documentation matters most?

Answer

Record objective findings, functional limits, treatment provided, response and progress clearly and factually, as insurers and regulators may review them. Follow the relevant state scheme's rules on reports and approvals, and keep patient privacy.

#61 · Musculoskeletal Physiotherapy

What clinical features support a cervicogenic headache over migraine or tension-type headache?

Answer

Unilateral headache starting in the neck and spreading forward, provoked by neck movement or sustained posture, with reduced cervical range and tenderness of upper cervical joints. Reproduction of the headache with joint palpation supports the diagnosis.

#62 · Musculoskeletal Physiotherapy

What does the cervical flexion rotation test assess in a cervicogenic headache patient?

Answer

With the neck fully flexed, passive rotation tests C1-2 mobility. Marked restriction of rotation compared with the other side, often with headache reproduction, suggests upper cervical dysfunction. It helps select and monitor treatment.

#63 · Musculoskeletal Physiotherapy

Which headache features are red flags needing urgent medical review?

Answer

Sudden severe thunderclap onset, headache with fever and neck stiffness, new neurological deficit, visual change, headache after head trauma, or a first headache in an older patient need urgent referral. Do not treat these in physiotherapy first.

#64 · Musculoskeletal Physiotherapy

Why should you screen for vertebrobasilar insufficiency before upper cervical manual therapy?

Answer

Signs such as dizziness, diplopia, dysarthria, dysphagia, drop attacks, nausea, nystagmus and numbness (the 5 Ds and 3 Ns) may indicate vascular compromise. Positive findings are a precaution and contraindication to manipulation, so refer for medical review.

#65 · Musculoskeletal Physiotherapy

What exercise approach has good evidence for managing cervicogenic headache?

Answer

Combine deep neck flexor training, scapular and postural strengthening and graded endurance work, with manual therapy as an adjunct. Active exercise gives lasting benefit and builds self-management, and patients should be taught a home programme.

#66 · Musculoskeletal Physiotherapy

A patient with cervicogenic headache wants an interpreter for sessions. How do you proceed?

Answer

Book a qualified interpreter, such as through the Translating and Interpreting Service, rather than relying on family. Accurate understanding is needed for informed consent, history taking and home exercise instructions. Record that an interpreter was used.

#67 · Musculoskeletal Physiotherapy

Which findings on examination support carpal tunnel syndrome?

Answer

Night-time paraesthesia in the thumb, index, middle and radial half of the ring finger, with positive Phalen's or Tinel's signs and thenar weakness or wasting in advanced cases. Sparing of the palm skin sensation helps distinguish it from proximal lesions.

#68 · Musculoskeletal Physiotherapy

Why does the palm usually keep normal sensation in carpal tunnel syndrome?

Answer

The palmar cutaneous branch of the median nerve leaves before the carpal tunnel, so it is not compressed. Altered sensation over the thenar eminence points instead to a more proximal median nerve problem or a cervical source.

#69 · Musculoskeletal Physiotherapy

What conservative treatment is first line for mild to moderate carpal tunnel syndrome?

Answer

A neutral wrist night splint, activity and workstation modification, and nerve gliding exercises are standard first-line care. Corticosteroid injection by a doctor may also help in the short term. Refer if symptoms persist or weakness progresses.

#70 · Musculoskeletal Physiotherapy

Which carpal tunnel findings suggest the need for surgical referral?

Answer

Thenar muscle wasting, persistent numbness, constant weakness or failure of conservative care indicate significant axonal involvement. Refer to a GP or hand surgeon, as delay risks permanent nerve damage. Nerve conduction studies may confirm severity.

#71 · Musculoskeletal Physiotherapy

A worker with carpal tunnel syndrome has a workers compensation claim. What is your role?

Answer

Provide treatment, document work-related causes and functional capacity objectively, communicate with the case manager and employer within consent, and support graded return to suitable duties. Follow the state scheme's requirements for reports and approvals.

#72 · Musculoskeletal Physiotherapy

What is the usual initial management of a stable proximal humerus fracture treated non-operatively?

Answer

Sling immobilisation for comfort for a short period under medical direction, then early gentle pendulum and passive movement as the surgeon allows, progressing to active and strengthening phases once healing is evident on imaging.

#73 · Musculoskeletal Physiotherapy

Which neurovascular check is vital after a proximal humerus fracture?

Answer

Assess axillary nerve function (deltoid contraction and lateral shoulder sensation), distal pulses, capillary refill and hand sensation. The axillary nerve and axillary artery lie close to the surgical neck, so new deficits need urgent medical review.

#74 · Musculoskeletal Physiotherapy

Why should rotator cuff strengthening be delayed after a proximal humerus fracture?

Answer

Tuberosity fragments carry the cuff tendon attachments, so early resisted contraction can displace fragments and impair union. Start active-assisted movement only when healing is confirmed and the surgeon clears the patient, then add resistance in stages.

#75 · Musculoskeletal Physiotherapy

An older patient with a proximal humerus fracture lives alone. What discharge and funding considerations apply?

Answer

Assess falls risk, daily task independence and home supports, and consider referral to My Aged Care for services such as home help. Provide sling care and one-handed task teaching, and involve family, with the patient's consent.

#76 · Musculoskeletal Physiotherapy

Why is early mobilisation, often day 1 after hip fracture surgery, a priority for the physiotherapist?

Answer

Early out-of-bed mobilisation reduces complications such as chest infection, pressure injury, VTE and deconditioning, and supports return to prior function. Weight bearing is guided by the surgeon's orders and the fixation type.

#77 · Musculoskeletal Physiotherapy

After a hemiarthroplasty for neck of femur fracture, what hip precautions might apply and who decides them?

Answer

The surgeon decides, usually based on the surgical approach (for example posterior approach: avoid excessive flexion, adduction and internal rotation). Check the operative notes and orders, and teach the patient clear, practical positioning advice.

#78 · Musculoskeletal Physiotherapy

A patient on day 2 after hip fracture surgery becomes acutely confused and drowsy. What is your action?

Answer

Suspect delirium, which is common after hip fracture and can signal infection, pain, hypoxia or medication effects. Hold the session, check observations, and escalate promptly to the medical or nursing team.

#79 · Musculoskeletal Physiotherapy

Which outcome measure suits a ward patient after hip fracture to track transfers and walking progress?

Answer

The Timed Up and Go measures functional mobility and falls risk, and the De Morton Mobility Index suits acute older patients. Repeat it to show change and guide walking aid choice and discharge planning.

#80 · Musculoskeletal Physiotherapy

An older patient with a neck of femur fracture has dementia and cannot consent to treatment. How do you proceed?

Answer

Assess capacity for each decision. If lacking, involve the substitute decision-maker per state law, use simple communication, and act in the patient's best interests. Document the process and the discussion with family.

#81 · Musculoskeletal Physiotherapy

What are the key red flags to monitor for after rib fractures?

Answer

Watch for worsening breathlessness, falling oxygen saturation, rising respiratory rate, increasing pain limiting breathing, and signs of pneumothorax or haemothorax. Escalate urgently, as pulmonary complications drive morbidity after rib fractures.

#82 · Musculoskeletal Physiotherapy

Why is effective analgesia central to physiotherapy for a patient with multiple rib fractures?

Answer

Pain restricts deep breathing and coughing, leading to atelectasis, sputum retention and pneumonia. Liaise with the medical team to optimise analgesia so the patient can mobilise, breathe deeply and clear secretions.

#83 · Musculoskeletal Physiotherapy

What breathing exercises help prevent chest complications after rib fractures?

Answer

Teach regular deep breathing, incentive spirometry if available, and supported coughing by splinting the chest with a folded towel or pillow. Do these frequently each hour while awake, timed with analgesia.

#84 · Musculoskeletal Physiotherapy

A patient with rib fractures has a pulse oximeter reading falling and a flail segment moving paradoxically. What does this suggest?

Answer

Paradoxical movement of a segment indicates a flail chest, a serious injury impairing ventilation. Stop treatment, give oxygen per protocol, and call the medical emergency team urgently, as it may need ventilatory support.

#85 · Musculoskeletal Physiotherapy

What advice do you give an older outpatient with isolated rib fractures about return to activity?

Answer

Encourage gentle walking and normal daily movement within pain limits, keep up deep breathing, and avoid heavy lifting and contact activity until pain settles. Advise that healing usually takes around six weeks.

#86 · Musculoskeletal Physiotherapy

A rib fracture patient reports pain and swelling in one calf. Why must you act on this?

Answer

Reduced mobility raises the risk of deep vein thrombosis, and calf pain with swelling needs urgent medical assessment because a clot may travel to the lungs. Do not massage the limb and report the findings.

#87 · Musculoskeletal Physiotherapy

What are the main goals of physiotherapy in the first days after total knee replacement?

Answer

Control pain and swelling, regain knee extension and flexion, activate the quadriceps, mobilise early with an aid, and prevent complications. Full extension early matters because lost extension is hard to regain and causes a limp.

#88 · Musculoskeletal Physiotherapy

Why is regaining full knee extension emphasised early after total knee replacement?

Answer

A fixed flexion deformity increases quadriceps effort in walking, causes a limp and strains the hip and back. Position the knee straight at rest, avoid pillows behind the knee, and practise extension exercises regularly.

#89 · Musculoskeletal Physiotherapy

A patient after total knee replacement has a hot, red, swollen knee with fever at two weeks. What do you do?

Answer

Suspect a wound or joint infection, which is a serious complication. Stop active treatment, record observations and escalate to the surgical team urgently rather than continuing the exercise programme.

#90 · Musculoskeletal Physiotherapy

Which outcome measures track progress after total knee replacement?

Answer

Use goniometry for knee flexion and extension range, the Timed Up and Go for mobility, and the 30 second sit to stand for lower limb function. Compare repeated scores to set goals and justify discharge or progression.

#91 · Musculoskeletal Physiotherapy

A patient reaches 90 degrees knee flexion at six weeks after total knee replacement with a hard end feel. What is the concern?

Answer

Poor range at this stage suggests arthrofibrosis or stiffness and needs early review. Discuss with the surgeon, as manipulation under anaesthetic may be considered, and continue range exercises within the surgeon's guidance.

#92 · Musculoskeletal Physiotherapy

When can you delegate part of a post-knee-replacement exercise programme to an allied health assistant?

Answer

You may delegate a stable, defined programme once you have assessed the patient, given clear instructions and set review points. The physiotherapist stays responsible and must supervise and reassess, not delegate assessment or clinical decisions.

#93 · Musculoskeletal Physiotherapy

What is the first-line management for knee osteoarthritis according to current guidelines?

Answer

Education, exercise and weight management when relevant form the core of care for knee osteoarthritis. These should come before injections or surgery, and exercise suits people of all ages and disease severity.

#94 · Musculoskeletal Physiotherapy

What types of exercise are recommended for people with knee osteoarthritis?

Answer

Strengthening (especially quadriceps), aerobic and neuromuscular exercise all help pain and function. Choose what the patient enjoys and can sustain, and progress gradually, as adherence drives long-term benefit.

#95 · Musculoskeletal Physiotherapy

A patient fears exercise will wear out their arthritic knee. How do you respond?

Answer

Explain that osteoarthritis is not simple wear and tear and that appropriate exercise is safe, nourishes the joint and reduces pain. Mild discomfort during exercise is acceptable, and this education improves adherence.

#96 · Musculoskeletal Physiotherapy

A knee osteoarthritis patient has new sudden locking after a twist and cannot straighten the knee. What does this suggest?

Answer

A true mechanical block suggests a displaced meniscal tear or loose body, which is not typical osteoarthritis. Refer for medical and imaging review, and avoid forcing extension.

#97 · Musculoskeletal Physiotherapy

What is the role of weight loss in managing knee osteoarthritis for a patient with overweight?

Answer

Weight loss reduces joint load and improves pain and function, and combining it with exercise gives greater benefit. Discuss it sensitively, set realistic goals and involve a dietitian or GP where appropriate.

#98 · Musculoskeletal Physiotherapy

A patient with hip osteoarthritis has groin pain, reduced internal rotation and morning stiffness under 30 minutes. What does this pattern support?

Answer

Groin pain with limited internal rotation and short-lived stiffness is typical of hip osteoarthritis. Confirm with history and examination, and consider imaging or referral if red flags or an atypical course appear.

#99 · Musculoskeletal Physiotherapy

What is a key education message for a person newly diagnosed with hip osteoarthritis?

Answer

Explain that the condition is common, usually manageable and not inevitably progressive to surgery, and that staying active with exercise reduces pain. Offer self-management strategies and give clear expectations to reduce fear.

#100 · Musculoskeletal Physiotherapy

When would you refer a patient with hip osteoarthritis for orthopaedic opinion?

Answer

Refer when pain, function loss or night pain persist despite a good trial of education, exercise and weight management. Surgery is considered after conservative care, and the patient's goals and preferences guide this.

#101 · Musculoskeletal Physiotherapy

A 74-year-old woman has sudden mid-thoracic pain after bending to lift a pot. What does this presentation raise suspicion of?

Answer

Suspect an osteoporotic vertebral compression fracture. Typical features are sudden onset after minor load, focal spinal tenderness and pain on movement. Refer for imaging via the GP before starting spinal loading or manual techniques.

#102 · Musculoskeletal Physiotherapy

Why should you avoid sustained or loaded spinal flexion exercises in a patient with osteoporosis?

Answer

Flexion loads the anterior vertebral bodies, which is where most compression fractures occur. Teach neutral spine hip-hinge patterns, back extensor strengthening and safe lifting instead of sit-ups, toe touches or forward-bending stretches.

#103 · Musculoskeletal Physiotherapy

What exercise types are recommended to maintain bone density in an older adult with osteoporosis who has no acute fracture?

Answer

Progressive resistance training plus weight-bearing impact appropriate to the person's tolerance, with balance training added. Bone responds to load, so progress gradually and tailor impact to fracture history and pain.

#104 · Musculoskeletal Physiotherapy

How does a fall-prevention component fit into physiotherapy for a patient with osteoporosis?

Answer

Fractures result from the combination of weak bone and falls, so address both. Include balance and strength training, a home hazard review, footwear advice and medication review via the GP to reduce fall risk.

#105 · Musculoskeletal Physiotherapy

A patient with a suspected vertebral fracture develops new leg weakness and urinary retention. What is the correct action?

Answer

These are red flags for spinal cord or cauda equina compromise. Stop treatment, arrange urgent transfer to the emergency department, and document findings and your escalation. Do not wait for a routine GP appointment.

#106 · Musculoskeletal Physiotherapy

On newborn hip screening, what do the Ortolani and Barlow tests assess for developmental dysplasia of the hip?

Answer

Ortolani reduces a dislocated hip with abduction and anterior lift; Barlow tests whether a located hip can be dislocated by posterior pressure. A positive clunk means urgent referral to a paediatric orthopaedic service.

#107 · Musculoskeletal Physiotherapy

What is the usual first-line management for an infant under six months diagnosed with developmental dysplasia of the hip?

Answer

A dynamic abduction brace such as the Pavlik harness, which holds the hips flexed and abducted so the femoral head sits in the acetabulum. It is applied and monitored by the orthopaedic team; parents need clear handling education.

#108 · Musculoskeletal Physiotherapy

What advice should you give parents about nappy changing and carrying with an infant who has hip dysplasia?

Answer

Keep the hips in a natural flexed, abducted position. Avoid tight swaddling that straightens and adducts the legs. Hip-healthy positioning supports joint development and follows the principle of keeping the head of femur centred.

#109 · Musculoskeletal Physiotherapy

Describe the foot deformity in congenital talipes equinovarus and the usual first-line treatment.

Answer

The foot is in plantarflexion (equinus), hindfoot varus, forefoot adduction and often cavus. First-line treatment is serial casting by the Ponseti method, usually followed by a foot abduction brace to prevent relapse.

#110 · Musculoskeletal Physiotherapy

Why is adherence to the foot abduction brace emphasised after Ponseti casting for talipes?

Answer

Relapse risk is high when bracing is not worn as prescribed through early childhood. Educate carers on the wearing schedule, skin checks and troubleshooting, and involve an interpreter if the family's language needs it.

#111 · Musculoskeletal Physiotherapy

A family needs an interpreter when you explain a brace plan for their infant. What is the correct approach?

Answer

Book an accredited interpreter, such as through the Translating and Interpreting Service, rather than using a family member. Speak to the parents, use short sentences, check understanding with teach-back, and document the interpreter used.

#112 · Musculoskeletal Physiotherapy

What characterises juvenile idiopathic arthritis in terms of onset and joint findings?

Answer

Arthritis beginning before 16 years, lasting at least six weeks, with joint swelling, warmth, stiffness and reduced range, often worse after rest. Morning stiffness and reluctance to use a limb are common presenting features.

#113 · Musculoskeletal Physiotherapy

What exercise approach suits a child with JIA during a flare of a knee joint?

Answer

Gentle active range of motion and isometrics within comfort, with positioning to prevent flexion contracture. Avoid forceful stretching or heavy loading of the inflamed joint, and resume graded activity as the flare settles.

#114 · Musculoskeletal Physiotherapy

Why should children with JIA be encouraged to stay physically active between flares?

Answer

Activity maintains muscle strength, bone health and function, and reduces deconditioning and fatigue. Rest alone causes weakness and contracture, so promote low-impact, enjoyable activity such as swimming and cycling, graded to symptoms.

#115 · Musculoskeletal Physiotherapy

A child with JIA reports new eye redness, pain or light sensitivity. What should you do?

Answer

Uveitis is a recognised complication, particularly in oligoarticular JIA, and can cause vision loss if untreated. Refer promptly to the treating doctor or ophthalmology, and document the referral. Screening is usually coordinated by the rheumatology team.

#116 · Musculoskeletal Physiotherapy

How can you work with a school to support a child with JIA?

Answer

With parental consent, liaise with teachers about pacing, rest breaks, seating, carrying bags and modified physical education. Providing a written plan supports participation. Share only the information needed, respecting privacy.

#117 · Musculoskeletal Physiotherapy

What is the Beighton score used for, and how is a positive score generally interpreted?

Answer

It screens generalised joint hypermobility using nine points: passive little finger extension beyond 90 degrees (each side), thumb to forearm (each side), elbow hyperextension beyond 10 degrees (each side), knee hyperextension beyond 10 degrees (each side), and palms flat to floor with knees straight. Higher scores suggest generalised hypermobility, but it does not diagnose a syndrome alone.

#118 · Musculoskeletal Physiotherapy

What is the core physiotherapy approach for symptomatic joint hypermobility?

Answer

Progressive strengthening and motor control training, with proprioceptive work, to dynamically stabilise joints. Avoid end-range stretching and passive manipulation of lax joints. Teach joint protection and pacing, and graded return to activity.

#119 · Musculoskeletal Physiotherapy

Why is end-range stretching generally avoided in people with generalised hypermobility and pain?

Answer

Their joints already move beyond normal range, and stretching adds stress to lax passive restraints without addressing the actual deficit, which is control. Focus on strength through inner and mid range, plus stability training.

#120 · Musculoskeletal Physiotherapy

A hypermobile patient reports fainting, palpitations and widespread skin fragility with easy bruising. What should you consider?

Answer

These features may suggest a connective tissue disorder such as hypermobile or vascular Ehlers-Danlos syndrome, or autonomic involvement. Refer to the GP for assessment, since some types carry vascular risk. Do not apply forceful techniques.

#121 · Musculoskeletal Physiotherapy

What does a positive apprehension test at the shoulder indicate?

Answer

Pain or a feeling of impending dislocation with the arm in abduction and external rotation, relieved by a posterior force (relocation), suggests anterior glenohumeral instability. It guides rehabilitation focus and the need for surgical opinion.

#122 · Musculoskeletal Physiotherapy

What structure is commonly injured, and which nerve checked, after an anterior shoulder dislocation?

Answer

The anteroinferior labrum (Bankart lesion) is commonly damaged. Check the axillary nerve, testing sensation over the lateral upper arm and deltoid activation, before and after reduction, and document findings. Report any neurovascular deficit urgently.

#123 · Musculoskeletal Physiotherapy

What is the usual early rehabilitation sequence after a first-time anterior shoulder dislocation managed non-surgically?

Answer

A short period of sling protection for comfort, then early pain-free movement, rotator cuff and scapular strengthening, and progression to proprioceptive and sport-specific work. Limit combined abduction and external rotation at end range early on.

#124 · Musculoskeletal Physiotherapy

Why do young active people after a first anterior shoulder dislocation need counselling about recurrence?

Answer

Recurrence risk is high in younger people, especially contact athletes. Explain that rehabilitation improves dynamic stability, and refer to an orthopaedic surgeon if there are recurrent episodes, a large bony defect or functional failure.

#125 · Musculoskeletal Physiotherapy

Which red flag features in a person with low back pain should prompt urgent medical referral for possible cauda equina syndrome?

Answer

New urinary retention or loss of bladder or bowel control, saddle numbness, progressive bilateral leg weakness or sexual dysfunction. Do not treat; arrange same day emergency assessment.

#126 · Musculoskeletal Physiotherapy

What does central sensitisation mean in a person with persistent low back pain, and how does it change your explanation to them?

Answer

The nervous system amplifies pain signals, so pain can exceed tissue damage. Explain that hurt does not always equal harm, using pain neuroscience education to reduce fear and support graded activity rather than rest.

#127 · Musculoskeletal Physiotherapy

Which findings on assessment suggest a persistent low back pain presentation is driven by central sensitisation?

Answer

Pain disproportionate to injury, widespread or shifting distribution, allodynia, sensitivity to light touch or stress, and poor response to mechanical loading. These guide you toward education and graded exposure, not more passive treatment.

#128 · Musculoskeletal Physiotherapy

What is the role of graded activity or graded exposure in persistent low back pain with fear avoidance?

Answer

Time-based or goal-based progression of feared movements and activities, not pain-contingent pacing. It rebuilds confidence and capacity, and reduces avoidance, which keeps pain and disability going.

#129 · Musculoskeletal Physiotherapy

Why would you use the STarT Back tool or a similar screening tool for a patient with ongoing low back pain?

Answer

It stratifies risk of poor outcome using psychosocial and physical items, so you can match care intensity. Higher risk patients need a psychologically informed approach, not just exercise or manual therapy.

#130 · Musculoskeletal Physiotherapy

A patient with persistent low back pain asks you to book weekly passive treatment indefinitely. How do you respond professionally?

Answer

Explain the evidence favours active management, self-management and education over open-ended passive care. Set shared goals, agree a review point and a discharge plan, and avoid fostering dependence or unnecessary billing.

#131 · Musculoskeletal Physiotherapy

What clinical features support a diagnosis of temporomandibular dysfunction on examination?

Answer

Jaw pain with chewing or yawning, clicking or locking, restricted or deviated mouth opening, and tenderness of masticatory muscles or the joint. Opening of about three fingers (roughly 35 to 40 mm or more) is generally considered functional, and less suggests restriction.

#132 · Musculoskeletal Physiotherapy

Which red flags in a patient with jaw pain should prompt urgent medical referral rather than TMD treatment?

Answer

Sudden trismus with fever or swelling suggesting infection, a history of trauma with suspected fracture, unexplained weight loss or a mass, or new neurological signs. Refer promptly to the GP or emergency department.

#133 · Musculoskeletal Physiotherapy

What self-management advice would you give a patient with acute TMD pain?

Answer

Soft diet, avoid wide yawning and gum chewing, limit parafunctional habits like clenching, use relaxed tongue-up, teeth-apart posture, and apply heat or gentle jaw movement within comfort. This reduces joint and muscle loading.

#134 · Musculoskeletal Physiotherapy

Why should you screen the cervical spine in a patient with TMD?

Answer

Cervical dysfunction and TMD commonly coexist and refer pain to the jaw, face and head. Assessing and treating neck posture and mobility can improve jaw symptoms and prevents treating the wrong source.

#135 · Musculoskeletal Physiotherapy

A patient with TMD is also seeing a dentist for a splint. How should you work with them?

Answer

Communicate with the dentist with the patient's consent, share findings, and coordinate advice so treatment is not conflicting. Document the consent given, as this involves sharing personal health information.

#136 · Musculoskeletal Physiotherapy

Which features of musculoskeletal pain should make you suspect a bone tumour or pathological fracture?

Answer

Constant, progressive, night or rest pain unrelieved by position, pain out of proportion to a minor injury, a history of cancer, unexplained weight loss, and a palpable mass or bony tenderness. Stop loading and escalate.

#137 · Musculoskeletal Physiotherapy

Which cancers commonly metastasise to bone and so raise suspicion in a patient with new musculoskeletal pain?

Answer

Breast, prostate, lung, kidney and thyroid cancers, plus multiple myeloma. A past history of any of these with new persistent bone pain is a red flag needing medical review and imaging before physiotherapy loading.

#138 · Musculoskeletal Physiotherapy

What should you avoid in treating a patient with suspected pathological fracture?

Answer

Avoid manual therapy techniques, end-range stretching, strong resisted loading and percussion over the site. Limit weight-bearing as advised and arrange urgent medical assessment, because loading may cause complete fracture.

#139 · Musculoskeletal Physiotherapy

A patient with known bone metastases wants to exercise. How do you approach this?

Answer

Liaise with the oncology team for imaging-based stability advice, then prescribe within limits with low-impact, pain-guided and gradual loading. Educate on fracture warning signs such as sudden worsening pain, and document the plan.

#140 · Musculoskeletal Physiotherapy

A patient with a suspected tumour asks you not to tell their family. What is your professional obligation?

Answer

Respect their confidentiality and capacity to decide who is told. Still communicate clearly the need for urgent medical review, document your advice, and share information only with consent unless there is a serious risk to others.

#141 · Musculoskeletal Physiotherapy

How do you classify a peripheral nerve injury using Seddon's classification, from mildest to most severe?

Answer

Neurapraxia (conduction block, full recovery), axonotmesis (axon damage, endoneurium intact, recovery depends on regeneration), then neurotmesis (complete disruption, needs surgical repair). It guides prognosis and referral.

#142 · Musculoskeletal Physiotherapy

What does Tinel's sign suggest after a peripheral nerve injury?

Answer

Tingling distal to a tapped point along the nerve indicates regenerating axons. An advancing Tinel's sign over weeks suggests ongoing regrowth, while a static one may point to a lack of progress and warrant review.

#143 · Musculoskeletal Physiotherapy

How should you protect a limb with sensory loss after nerve injury?

Answer

Teach daily skin checks, avoid heat, cold and sharp objects, use protective padding and splints, and manage oedema. This prevents burns, pressure injury and unnoticed wounds because protective sensation is reduced.

#144 · Musculoskeletal Physiotherapy

Why do you maintain passive range and use splinting in the early stage of a motor nerve injury?

Answer

To prevent contracture and joint stiffness while muscles are denervated, and to keep the limb in a position of function. Strengthening begins only once reinnervation allows active muscle contraction.

#145 · Musculoskeletal Physiotherapy

When should a patient with a suspected complete nerve transection after trauma be referred urgently?

Answer

As soon as it is suspected: a deep laceration with total loss of motor and sensory function does not recover unaided. Prompt referral to a surgeon matters because outcomes decline as time to nerve repair increases.

#146 · Musculoskeletal Physiotherapy

What features differentiate inflammatory back pain from mechanical back pain?

Answer

Onset before about 40 years, insidious onset, morning stiffness over 30 minutes, improvement with exercise but not rest, and night pain in the second half of the night. These suggest axial spondyloarthritis and need rheumatology referral.

#147 · Musculoskeletal Physiotherapy

What is the main goal of exercise in ankylosing spondylitis?

Answer

To maintain spinal mobility, posture, chest expansion and function. Regular daily extension, mobility and aerobic exercise slow stiffness and fixed flexion deformity, and are recommended alongside medical management.

#148 · Musculoskeletal Physiotherapy

Which outcome measure is commonly used to monitor disease activity in ankylosing spondylitis?

Answer

The Bath Ankylosing Spondylitis Disease Activity Index (BASDAI) is a patient-reported measure of fatigue, pain, and stiffness. Higher scores indicate more active disease and can prompt a medical review.

#149 · Musculoskeletal Physiotherapy

Why does a patient with long-standing ankylosing spondylitis who has a minor fall need careful assessment?

Answer

Fused, brittle spine segments are prone to fractures even from low-energy trauma, and these may be missed. Treat new pain or neurological signs as an emergency and refer for imaging rather than treating as a strain.

#150 · Musculoskeletal Physiotherapy

How do you advise a patient with ankylosing spondylitis about work and posture?

Answer

Encourage regular position changes, an ergonomic setup that avoids sustained flexion, a firm sleep surface with a low pillow, and breathing and chest expansion exercises. Involve an employer with the patient's consent.

#151 · Musculoskeletal Physiotherapy

Which findings help support pelvic girdle pain rather than a lumbar source in a pregnant woman?

Answer

Pain reproduced by posterior pelvic pain provocation tests, a painful or difficult active straight leg raise, and palpation tenderness over the pelvic joints, with no lumbar nerve root signs. This points to a pelvic origin.

#152 · Musculoskeletal Physiotherapy

A pregnant woman finds the active straight leg raise painful. What is the next step in the test?

Answer

Repeat it with manual compression across the pelvis. If it becomes easier, poor load transfer through the pelvis is likely, which supports trialling a pelvic support belt and stabilising exercise.

#153 · Musculoskeletal Physiotherapy

A woman with pelvic girdle pain reports fever, severe unremitting night pain and new saddle numbness. How do you respond?

Answer

These are red flags for infection or cauda equina compromise, not routine pelvic girdle pain. Stop treatment and escalate urgently to the obstetric team or emergency department the same day.

#154 · Musculoskeletal Physiotherapy

What daily activity advice reduces pelvic girdle pain in pregnancy?

Answer

Keep knees together when rolling in bed or leaving a car, use smaller steps, avoid prolonged single-leg standing and asymmetrical loading, and sleep with a pillow between the knees. This limits shear through the pelvic joints.

#155 · Musculoskeletal Physiotherapy

How should you involve the maternity care team when pelvic girdle pain limits a woman's mobility late in pregnancy?

Answer

With her consent, liaise with the midwife or obstetrician about pain, mobility aids and comfortable birthing positions. Document what was shared and her informed choices, as privacy rules apply to pregnancy information.

#156 · Musculoskeletal Physiotherapy

What pattern of leakage defines stress urinary incontinence, and what causes it?

Answer

Leakage with raised abdominal pressure such as coughing, sneezing, lifting or jumping, without a strong urge beforehand. It reflects inadequate urethral support or sphincter closure, often linked to pelvic floor weakness after childbirth.

#157 · Musculoskeletal Physiotherapy

What is first-line conservative treatment for stress urinary incontinence, and what makes it more effective?

Answer

Pelvic floor muscle training. It works best when individually taught and supervised, with correct technique confirmed, progressive loading and enough repetitions, rather than a written handout alone. Review adherence and progress regularly.

#158 · Musculoskeletal Physiotherapy

What does a correct pelvic floor contraction look like when you assess technique?

Answer

An inward and upward squeeze and lift around the urethra, vagina and anus, with no buttock clenching, breath holding or downward bearing. Confirm by observation, palpation or real-time ultrasound and teach feedback.

#159 · Musculoskeletal Physiotherapy

What consent steps are needed before an internal pelvic floor examination?

Answer

Explain the purpose and technique, obtain clear informed consent, offer a chaperone and an interpreter if needed, and say she may stop at any time. Document the consent, or the refusal and alternatives used.

#160 · Musculoskeletal Physiotherapy

A woman with incontinence reports visible blood in her urine. What is the correct action?

Answer

Haematuria is a red flag outside routine pelvic floor care. Refer promptly to her GP for investigation before or alongside continuing training, and document the finding and referral.

#161 · Musculoskeletal Physiotherapy

Which type of exercise has the strongest evidence for preventing falls in community-dwelling older adults?

Answer

Regular, challenging balance and functional exercise, progressed over time and sustained long term. Walking alone or low-challenge exercise is less effective, so progress balance difficulty by narrowing the base and reducing arm support.

#162 · Musculoskeletal Physiotherapy

What does the Timed Up and Go test measure, and how should you use the result?

Answer

The time to stand from a chair, walk three metres, turn, return and sit. Slower times suggest poorer mobility and higher falls risk, but use it to track change over time, not as a stand-alone falls predictor.

#163 · Musculoskeletal Physiotherapy

What principle guides strength training for a frail older person?

Answer

Progressive overload with supervision: start at a safe low load, increase gradually as strength improves, and monitor symptoms and technique. Resistance training safely improves strength and function even in very old and frail people.

#164 · Musculoskeletal Physiotherapy

How do you screen for orthostatic hypotension in an older person who has had a fall, and what do you do?

Answer

Measure blood pressure lying, then after standing for up to three minutes. A systolic drop of 20 mmHg or more, or diastolic drop of 10 mmHg or more, is significant. Refer to the GP for medication review.

#165 · Musculoskeletal Physiotherapy

What must you do when delegating a falls prevention exercise programme to an allied health assistant?

Answer

You remain accountable: assess and prescribe the programme, give clear instructions on limits, supervise and review progress. The assistant should stop and report new pain, dizziness or a fall. Document the delegation.

#166 · Musculoskeletal Physiotherapy

What do the Budapest criteria require for diagnosing complex regional pain syndrome?

Answer

Continuing pain disproportionate to the injury, symptoms in at least three categories (sensory, vasomotor, sudomotor or oedema, motor or trophic), signs in at least two, and no other diagnosis that better explains the picture.

#167 · Musculoskeletal Physiotherapy

What is the typical presentation of complex regional pain syndrome after a wrist fracture?

Answer

Pain out of proportion to the injury with allodynia, swelling, skin colour and temperature change, altered sweating and stiffness or weakness of the hand. It often follows fracture, surgery or prolonged immobilisation.

#168 · Musculoskeletal Physiotherapy

What are the three stages of graded motor imagery for complex regional pain syndrome?

Answer

First left and right laterality recognition, then imagined movements, then mirror therapy. The graded sequence activates movement representations without provoking pain, and each stage progresses only when the previous one is tolerated.

#169 · Musculoskeletal Physiotherapy

What is the guiding principle for exercise and activity in complex regional pain syndrome?

Answer

Encourage early, graded use of the limb within tolerable pain, with desensitisation and education, and avoid prolonged immobilisation or rest. Explain that pain does not mean harm to build confidence and function.

#170 · Musculoskeletal Physiotherapy

A worker develops complex regional pain syndrome after a compensable injury. What professional steps apply?

Answer

Document objective findings and function clearly, communicate with the case manager and treating doctor within consent, and recommend referral to a pain specialist or multidisciplinary team. Keep to the approved treatment plan.

#171 · Musculoskeletal Physiotherapy

How do you move a patient with an unstable cervical spine injury before it has been cleared?

Answer

Maintain neutral spinal alignment using a coordinated log roll with enough trained staff and a person controlling the head. Follow medical team precautions until imaging and a senior clinician clear the spine.

#172 · Musculoskeletal Physiotherapy

A patient with a thoracic fracture develops new leg weakness during mobilisation. What do you do?

Answer

Stop immediately, return the patient to a safe supported position, check vital signs, and escalate urgently to the medical team. New neurological deficit suggests worsening cord compromise and needs immediate review.

#173 · Musculoskeletal Physiotherapy

A patient with a T4 spinal cord injury suddenly has severe headache, flushing and a very high blood pressure. What is this?

Answer

Autonomic dysreflexia, a medical emergency. Sit the patient upright, loosen clothing, check for a blocked catheter or bowel or skin triggers, and call for urgent medical help. Monitor blood pressure frequently.

#174 · Musculoskeletal Physiotherapy

Why do patients with a high cervical spinal cord injury need respiratory physiotherapy?

Answer

Paralysed abdominal and intercostal muscles weaken cough and reduce vital capacity, so secretions are retained. Use positioning, manually assisted cough and monitoring of lung function and fatigue, and escalate if ventilation deteriorates.

#175 · Musculoskeletal Physiotherapy

A trauma patient with limited English must follow spinal precautions. How do you communicate this?

Answer

Book an accredited interpreter, in person or by phone, rather than relying on family members. Check understanding using teach-back, and document the interpreter used and the patient's consent to the plan.

#176 · Neurological Physiotherapy

Why is very early, high-dose mobilisation within 24 hours of stroke onset not routinely recommended?

Answer

Very early mobilisation within 24 hours, at high dose, was linked to poorer outcomes in trials. Start mobilising early, after 24 hours when medically stable, in short, frequent sessions, and follow the medical team's guidance.

#177 · Neurological Physiotherapy

Which blood pressure and neurological changes during acute stroke mobilisation should make you stop and call the medical team?

Answer

Stop for new or worsening weakness, speech change, drowsiness, severe headache, vomiting, marked drop in blood pressure or symptomatic hypotension. These may signal extension of the stroke or poor perfusion, so return the patient to a safe position and escalate.

#178 · Neurological Physiotherapy

What is the main reason to position a patient with acute ischaemic stroke carefully in bed?

Answer

Good positioning protects the hemiparetic shoulder, limits skin damage, supports breathing and reduces abnormal tone. Change position regularly, support the affected limbs, and follow the team's head position orders, as lying flat may be ordered early.

#179 · Neurological Physiotherapy

What is the purpose of the FAST message in stroke and why does it matter to physiotherapists?

Answer

FAST (face, arms, speech, time) helps recognise stroke quickly. A patient with sudden new deficits during a session needs urgent medical review and an ambulance if in the community, because time to treatment limits brain damage.

#180 · Neurological Physiotherapy

A stroke patient on an acute ward needs a consent decision but has aphasia. How do you obtain valid consent for physiotherapy?

Answer

Assess capacity for the specific decision, use supported communication such as pictures, gesture, short sentences and extra time, and involve a speech pathologist or interpreter. If capacity is lacking, follow the substitute decision-maker process and document it.

#181 · Neurological Physiotherapy

What is the evidence-based principle for improving arm function after stroke?

Answer

Repetitive, task-specific, goal-directed practice at sufficient intensity drives recovery. Practise real functional tasks such as reaching and grasping, with many repetitions and progressive challenge, rather than passive movement or isolated strengthening alone.

#182 · Neurological Physiotherapy

What does constraint-induced movement therapy involve and who suits it?

Answer

It restricts use of the unaffected hand for much of the day while intensively practising tasks with the affected hand. It suits people with some active wrist and finger extension, adequate cognition and balance, and willingness to take part.

#183 · Neurological Physiotherapy

Which outcome measure assesses upper limb activity after stroke through grasp, grip, pinch and gross movement?

Answer

The Action Research Arm Test measures arm and hand function across grasp, grip, pinch and gross movement. Repeat it to track change, set realistic goals and show whether the programme is working.

#184 · Neurological Physiotherapy

Why should you avoid forcing range or using overhead pulleys for a flaccid hemiparetic arm?

Answer

A flaccid shoulder lacks muscle support, so forced or pulley-assisted elevation can stretch and injure the capsule and rotator cuff, causing pain and subluxation. Use careful handling, supported positioning and active-assisted movement within pain-free range.

#185 · Neurological Physiotherapy

How can you involve a person with hemiparetic arm weakness at home while you are not present, and who may help?

Answer

Set a written or pictorial self-practice programme with carers or family trained to supervise, and consider an allied health assistant for supervised repetition. You remain responsible for assessment, prescription, progression and review of the delegated task.

#186 · Neurological Physiotherapy

What is unilateral spatial neglect and how does it differ from hemianopia?

Answer

Neglect is failure to attend to or respond to one side of space despite intact sensation and vision, usually left after a right hemisphere stroke. Hemianopia is a field loss that the person can compensate for by scanning, whereas neglect reflects reduced awareness.

#187 · Neurological Physiotherapy

Name a simple bedside test for spatial neglect and what a positive result looks like.

Answer

Line bisection: the person marks the middle of a horizontal line and, with neglect, places the mark well toward the unaffected side. Cancellation tasks and clock drawing are also used, and results guide safety planning and treatment.

#188 · Neurological Physiotherapy

What is a core treatment principle for neglect during functional mobility?

Answer

Cue attention toward the neglected side during real tasks, arrange the environment so important items and people are placed on the neglected side, and teach scanning strategies. Graded cues are reduced as awareness improves.

#189 · Neurological Physiotherapy

Why does unilateral neglect raise falls risk and affect discharge planning?

Answer

The person may collide with objects, miss hazards on one side and underestimate disability, with poor insight. It lowers safety in walking, transfers and wheelchair use, so you need supervision, environmental modification, carer education and a home assessment.

#190 · Neurological Physiotherapy

A patient with neglect keeps leaving the neglected arm hanging off the wheelchair. What should you do?

Answer

Position and support the arm on a tray or armrest, check it at transfers, and teach the person and carers to look for it. This prevents injury and shoulder pain, and links to attention training.

#191 · Neurological Physiotherapy

What is the risk of an inaccurate sense of ability after stroke with neglect or poor insight when setting goals?

Answer

The person may overestimate their abilities and attempt unsafe tasks. Use graded, supervised practice, give honest feedback, involve the family in goal setting, and set measurable goals focused on safety and function.

#192 · Neurological Physiotherapy

Which outcome measure is commonly used to track functional independence after stroke in rehabilitation?

Answer

The Functional Independence Measure rates assistance needed across self-care, mobility and cognition. It tracks change through admission and discharge, supports goal setting and funding documentation, and helps plan the level of ongoing support.

#193 · Neurological Physiotherapy

How should you position and handle a hemiplegic shoulder to reduce risk of subluxation?

Answer

Support the arm on pillows or a table, avoid pulling on the affected arm during transfers, and handle by supporting the scapula and humerus. Maintain scapular alignment and avoid unsupported dependent hanging.

#194 · Neurological Physiotherapy

What is the usual cause of shoulder subluxation after stroke and what does it feel like on palpation?

Answer

Weak shoulder girdle muscles and a flaccid arm lose support for the humeral head, so a palpable gap appears below the acromion. It is often painless at first, but traction injury can then cause pain.

#195 · Neurological Physiotherapy

Which conditions should you consider in a hemiplegic patient with a swollen, painful, stiff hand and shoulder?

Answer

Complex regional pain syndrome should be considered, with swelling, pain, colour and temperature change and stiffness. Refer promptly to medical staff, as early management improves outcome, and avoid aggressive stretching.

#196 · Neurological Physiotherapy

What is the role of strapping or taping for a subluxed hemiplegic shoulder?

Answer

Taping or a support may reduce subluxation and pain in the short term while muscle activity returns, but it must not restrict circulation or skin or encourage immobility. Combine with active retraining and monitor the skin.

#197 · Neurological Physiotherapy

A stroke patient with shoulder pain asks to stop therapy. What is the right approach on consent?

Answer

The person may withdraw consent at any time. Stop, explore the pain, reassess, explain risks and options, adjust the programme, and document the discussion. Do not push through pain, and refer if pain is severe.

#198 · Neurological Physiotherapy

Why is haemorrhagic stroke managed differently from ischaemic stroke in the early phase, regarding physiotherapy?

Answer

Bleeding can expand and raise intracranial pressure, so activity is more cautious and guided by medical orders and blood pressure targets. Follow neurosurgical or stroke team instructions closely and avoid exertion or straining until cleared.

#199 · Neurological Physiotherapy

What are the signs of rising intracranial pressure that require you to stop and call for urgent help?

Answer

Worsening headache, vomiting, falling level of consciousness, new confusion, pupil change, slowed heart rate with rising blood pressure and new focal deficits. Stop treatment, keep the person safe and alert the medical team immediately.

#200 · Neurological Physiotherapy

Which activities should you avoid with raised intracranial pressure risk and why?

Answer

Avoid straining, breath holding (Valsalva), head-down positioning and sustained high-effort exercise, as these raise intracranial pressure. Keep the head elevated if ordered, use gentle movement, and follow the medical orders and monitoring parameters.

#201 · Neurological Physiotherapy

What three responses does the Glasgow Coma Scale score, and why does a falling score matter during physiotherapy after traumatic brain injury?

Answer

It scores eye opening, verbal response and motor response. A falling score signals neurological deterioration, so stop treatment, make the patient safe and escalate to the medical team immediately.

#202 · Neurological Physiotherapy

A patient with severe traumatic brain injury has raised intracranial pressure. Which physiotherapy handling precautions apply?

Answer

Avoid prolonged head-down positions, sustained breath holding and straining, and cluster handling carefully. Keep the head neutral and monitor ICP and blood pressure against ward limits, because stimulation can spike pressure.

#203 · Neurological Physiotherapy

Why are positioning, splinting or serial casting started early after moderate to severe traumatic brain injury?

Answer

Prolonged immobility and spasticity cause soft tissue shortening and contracture, commonly at the ankle plantarflexors. Early positioning and stretching or casting preserve range and later function, and are easier than reversing fixed contracture.

#204 · Neurological Physiotherapy

A person with severe brain injury lacks capacity to consent to treatment. What should the physiotherapist do?

Answer

Identify the legally appointed substitute decision-maker under state or territory law, explain the plan and seek their consent. Involve the patient as far as possible, and document the capacity assessment and who consented.

#205 · Neurological Physiotherapy

Which Australian scheme types commonly fund long-term rehabilitation after a road-related traumatic brain injury, and what should you document?

Answer

Compulsory third party (CTP) or state transport accident schemes, or the NDIS for ongoing disability supports. Document clear functional goals and progress, since these funders approve services against measurable outcomes.

#206 · Neurological Physiotherapy

What does the Westmead Post-traumatic Amnesia Scale tell you, and how does it guide rehabilitation?

Answer

It tracks orientation and memory for new events to show whether post-traumatic amnesia has resolved. Until it clears, keep sessions short, repetitive and routine-based, because new learning is unreliable.

#207 · Neurological Physiotherapy

A patient in post-traumatic amnesia is agitated in the gym. What environmental changes help most?

Answer

Reduce stimulation: a quiet, uncluttered space, one familiar staff member, a calm voice and short consistent sessions. Agitation in this phase is part of recovery, so reduce triggers instead of confronting the patient.

#208 · Neurological Physiotherapy

Which tool measures the severity of agitation in acquired brain injury and helps you decide whether to proceed with therapy?

Answer

The Agitated Behaviour Scale rates observed behaviours such as restlessness and aggression. Higher scores mean you should modify or defer intensive therapy, and repeated scores show whether management is working.

#209 · Neurological Physiotherapy

When can an allied health assistant safely help with mobility practice for a patient recovering from post-traumatic amnesia?

Answer

When the physiotherapist has assessed the patient, written a clear program, and judged the risk low with supervision available. The AHA follows the program and reports changes, agitation or falls back to the physiotherapist.

#210 · Neurological Physiotherapy

A patient with brain injury hits a staff member during treatment and a slip occurs in the gym. What must you do afterwards?

Answer

Make everyone safe, then report the event through the facility's incident reporting system promptly and factually. Document what happened in the notes, review risk and update the care plan with the team.

#211 · Neurological Physiotherapy

After brain tumour resection, which new symptoms are red flags to report urgently?

Answer

A severe or worsening headache, repeated vomiting, a new seizure, reduced consciousness or new neurological deficit. They may indicate raised intracranial pressure, bleeding or swelling, so stop and escalate immediately.

#212 · Neurological Physiotherapy

A patient has a seizure during gait practice after tumour resection. What is the correct first aid response?

Answer

Protect from injury by lowering the patient to a safe position and clearing hazards, cushion the head and note the time. Do not restrain or put anything in the mouth. Call for help and place on their side afterwards.

#213 · Neurological Physiotherapy

A patient on long-term corticosteroids after brain tumour surgery has trouble standing from a chair. What cause should you consider?

Answer

Steroid-induced proximal myopathy, causing hip and shoulder girdle weakness without sensory loss. Include proximal strengthening and functional sit-to-stand practice, and report the weakness so the medical team can review the steroid plan.

#214 · Neurological Physiotherapy

Why should visual field loss and neglect be screened early after resection of a parietal or occipital tumour?

Answer

A hemianopia or neglect affects safe mobility, falls risk and engagement. Screening guides positioning, scanning training and environmental set-up, and allows referral to occupational therapy or orthoptics for further assessment.

#215 · Neurological Physiotherapy

A patient with a brain tumour is having a long rehabilitation admission but a limited prognosis. How should goal setting be approached?

Answer

Set goals with the patient and family around what matters most, such as safe transfers or time at home. Review them regularly as status changes and coordinate with palliative care without withholding function-focused rehabilitation.

#216 · Neurological Physiotherapy

What are external cues in Parkinson disease, and why do they improve walking?

Answer

Visual cues such as floor lines, auditory cues such as a metronome, or tactile cues bypass the impaired automatic basal ganglia pathways. They use more conscious attentional control to normalise step length and rhythm.

#217 · Neurological Physiotherapy

A person with Parkinson disease freezes at a doorway. Which strategy is appropriate to teach?

Answer

Use a cue before the freeze: stepping over a line or object, counting or marching to a rhythm, or shifting weight side to side. Avoid pulling or rushing them, which can worsen freezing and risk a fall.

#218 · Neurological Physiotherapy

What is the purpose of LSVT BIG in Parkinson disease?

Answer

An intensive, amplitude-focused exercise programme that trains large, deliberate movements to counter hypokinesia and bradykinesia. It improves movement size, then mobility, and needs a trained clinician with high-intensity practice.

#219 · Neurological Physiotherapy

When is the best time to schedule physiotherapy for a person with Parkinson disease taking levodopa?

Answer

During their 'on' period, when medication is working and movement is easiest. Learn their medication times, note wearing-off patterns, and teach practice during on periods and safe strategies for off periods.

#220 · Neurological Physiotherapy

A person with Parkinson disease becomes dizzy on standing and nearly falls. What should you assess and teach?

Answer

Check for orthostatic hypotension with lying and standing blood pressure, as it is common in Parkinson disease. Teach slow position changes, hydration advice and report significant drops to the GP or neurologist.

#221 · Neurological Physiotherapy

A woman with multiple sclerosis feels weaker and more fatigued after a hot shower, then recovers when cool. What is this phenomenon?

Answer

Uhthoff phenomenon: a temporary worsening of symptoms with raised body temperature, caused by slowed conduction in demyelinated nerves. It is not a relapse, so cool the person and schedule exercise in cool conditions.

#222 · Neurological Physiotherapy

Which fatigue management strategies should a physiotherapist teach a person with multiple sclerosis?

Answer

Energy conservation: pacing, planned rests, prioritising tasks and spreading activity through the day. Pair this with regular submaximal exercise, which improves fitness and reduces fatigue, and avoid pushing to exhaustion.

#223 · Neurological Physiotherapy

A person with multiple sclerosis reports new weakness lasting over 24 hours that worsens over days, with no fever. What does this suggest?

Answer

A possible relapse, which needs prompt medical review rather than pushing through exercise. Advise contacting the neurologist or GP and adjust activity until assessed, since relapses may need specific treatment.

#224 · Neurological Physiotherapy

What does the Expanded Disability Status Scale (EDSS) measure in multiple sclerosis, and what is its limit?

Answer

It rates neurological impairment and disability across functional systems, with a strong weighting to walking ability. It is useful for describing disease progression but is not sensitive to fatigue or change in daily function.

#225 · Neurological Physiotherapy

Which funding pathway may support a younger Australian with multiple sclerosis who needs ongoing equipment and therapy?

Answer

The NDIS, if they meet access criteria for permanent disability. Write clear functional reports linking goals to supports such as exercise programs, mobility aids and home modifications, as the NDIA decides funding on reasonable and necessary supports.

#226 · Neurological Physiotherapy

Which symptoms suggest respiratory muscle weakness is developing in a person with motor neurone disease?

Answer

Orthopnoea, morning headache, daytime sleepiness, disturbed sleep, shallow breathing and a weak cough. Report these promptly to the medical and respiratory team, because they may signal the need for assessment for non-invasive ventilation.

#227 · Neurological Physiotherapy

Why is high-resistance or fatiguing exercise avoided in motor neurone disease, and what is prescribed instead?

Answer

Overwork can accelerate weakness in already damaged motor units. Prescribe low-load, submaximal activity that stops short of fatigue, plus energy conservation, and keep range and function in unaffected muscle groups.

#228 · Neurological Physiotherapy

What is the physiotherapy role when a person with motor neurone disease has a weak, ineffective cough?

Answer

Teach manual assisted cough and, where available, mechanical insufflation-exsufflation, and train carers in the techniques. Early secretion clearance reduces chest infection risk, which is a major cause of deterioration.

#229 · Neurological Physiotherapy

What does the ALSFRS-R measure, and how does a physiotherapist use it?

Answer

It is a 12-item functional rating scale (completed by interview or self-report) covering bulbar, fine motor, gross motor and respiratory function. Repeated scores track the rate of progression, which guides timing of equipment, respiratory referral and goals of care discussions.

#230 · Neurological Physiotherapy

How should a physiotherapist approach goals of care and refusal of ventilation for a person with motor neurone disease?

Answer

Support early advance care planning with the person's palliative care and medical team. A person with capacity may accept or refuse interventions such as ventilation, and their documented wishes must guide your treatment.

#231 · Neurological Physiotherapy

What does a positive Gowers sign indicate in a boy with Duchenne muscular dystrophy?

Answer

He uses his hands to climb up his thighs to stand, showing proximal lower limb and trunk weakness, especially hip and knee extensors. It reflects dystrophin deficiency in an X-linked condition.

#232 · Neurological Physiotherapy

What exercise principle applies to a child with Duchenne muscular dystrophy?

Answer

Use submaximal, low-resistance, functional activity and aquatic exercise, and avoid high-resistance and eccentric loading and exhaustion. Damaged dystrophin-deficient muscle is vulnerable to exercise-induced injury, so quality and rest matter.

#233 · Neurological Physiotherapy

Which contracture prevention strategies suit a child with Duchenne muscular dystrophy who is still walking?

Answer

Regular stretching of calf, hamstrings, hip flexors and iliotibial band, night ankle-foot orthoses and standing time. Preserving range prolongs walking and makes later positioning and equipment use easier.

#234 · Neurological Physiotherapy

Why does Duchenne muscular dystrophy require monitoring beyond the musculoskeletal system?

Answer

Respiratory muscle weakness and cardiomyopathy develop with age, and scoliosis often progresses after walking is lost. Refer to cardiology and respiratory services, and teach lung volume recruitment and assisted cough as needed.

#235 · Neurological Physiotherapy

What professional considerations apply when a teenager with Duchenne muscular dystrophy needs a powered wheelchair?

Answer

Involve the young person in decisions as well as parents, seek consent appropriately, and complete a detailed trial and funding application, often through the NDIS. Plan transition from paediatric to adult services early.

#236 · Neurological Physiotherapy

What does an ASIA Impairment Scale grade A mean after spinal cord injury?

Answer

A complete injury: no sensory or motor function preserved in the sacral segments S4 to S5. Classification uses the ISNCSCI examination and guides prognosis, goals and equipment planning.

#237 · Neurological Physiotherapy

Why is tenodesis grasp preserved in a person with complete C6 tetraplegia?

Answer

Wrist extension drives passive finger flexion when the long finger flexors are slightly tight, giving a functional grasp. Do not stretch finger flexors fully with the wrist extended, or the grip is lost.

#238 · Neurological Physiotherapy

What respiratory problems are expected in acute complete high tetraplegia, and how does physiotherapy respond?

Answer

Weak diaphragm and absent abdominal and intercostal muscles reduce cough and vital capacity. Use assisted cough, positioning, secretion clearance and an abdominal binder in sitting, and liaise with the ventilation team.

#239 · Neurological Physiotherapy

How does a physiotherapist help prevent pressure injuries in a person with complete tetraplegia?

Answer

Teach regular repositioning, pressure relief, daily skin checks and correct cushion and seating setup. Insensate skin cannot signal damage, so education of the person and carers is the key preventive measure.

#240 · Neurological Physiotherapy

Who may fund equipment and ongoing therapy after a traumatic spinal cord injury in Australia?

Answer

The NDIS, and state or territory compensable schemes such as motor accident or workers compensation, may apply. Clear, detailed documentation of function and equipment needs supports funding decisions.

#241 · Neurological Physiotherapy

At what spinal cord injury level is autonomic dysreflexia a risk, and what causes it?

Answer

Injury at about T6 or above. A noxious stimulus below the lesion, such as a full bladder, triggers uncontrolled sympathetic reflex activity and a sudden rise in blood pressure.

#242 · Neurological Physiotherapy

List the typical signs of autonomic dysreflexia.

Answer

Sudden severe hypertension, pounding headache, sweating and flushing above the lesion, goosebumps, nasal congestion and a slow heart rate. It is a medical emergency because untreated hypertension can cause stroke.

#243 · Neurological Physiotherapy

What is the immediate physiotherapy response if autonomic dysreflexia occurs during treatment?

Answer

Sit the person upright with legs lowered, loosen tight clothing, monitor blood pressure and look for the trigger, checking the catheter, bladder and bowel. Call for urgent medical help if blood pressure stays high.

#244 · Neurological Physiotherapy

How should a physiotherapist manage orthostatic hypotension when sitting up or standing a person with tetraplegia?

Answer

Progress position gradually using a tilt table or reclining wheelchair, and use an abdominal binder and compression stockings. If symptoms or a blood pressure drop occur, return to lying flat and monitor recovery.

#245 · Neurological Physiotherapy

How should autonomic dysreflexia be handled when delegating care to an allied health assistant or carer?

Answer

Teach them the warning signs, the emergency steps and who to call, and ensure the person carries an emergency plan card. Incidents must be reported and documented according to local policy.

#246 · Neurological Physiotherapy

What does the Gross Motor Function Classification System describe for a child with cerebral palsy?

Answer

Five levels of self-initiated movement and mobility, from level I walking without limitation to level V transported in a wheelchair. It describes current ability and is not an outcome measure of change.

#247 · Neurological Physiotherapy

What mobility equipment is typical for a child at GMFCS level III?

Answer

Walking with a handheld mobility device such as a frame or crutches, often indoors, and wheeled mobility for longer community distances. Match equipment to the setting and review as the child grows.

#248 · Neurological Physiotherapy

Why is hip surveillance important in children with cerebral palsy, and how does GMFCS relate?

Answer

Hip migration risk rises with higher GMFCS level and can lead to pain and dislocation. Follow routine hip surveillance with imaging and refer early to orthopaedics when migration is found.

#249 · Neurological Physiotherapy

What does the Gross Motor Function Measure assess in a child with cerebral palsy?

Answer

Gross motor function across lying, sitting, crawling, standing and walking. It is sensitive to change over time, so it is used to evaluate the effect of therapy and goal progress.

#250 · Neurological Physiotherapy

How should goal setting and cultural safety be applied with an Aboriginal child with cerebral palsy and their family?

Answer

Set family-centred goals, listen to the family's priorities, involve Elders or community members as the family wishes, and consider travel and Country. Build trust and use plain language rather than assuming.

#251 · Neurological Physiotherapy

At what point should an infant who cannot roll or bring hands to midline be referred for developmental review?

Answer

Refer early when milestones are clearly delayed for corrected age, or when asymmetry, persistent primitive reflexes or loss of skills appear. Early referral to a paediatrician beats waiting, because early intervention improves outcomes.

#252 · Neurological Physiotherapy

Which observation in a young infant is a red flag for regression rather than simple delay?

Answer

Loss of previously gained motor skills is a red flag. It suggests a progressive neurological, metabolic or neuromuscular condition, so escalate urgently to the GP or paediatrician instead of simply starting a home programme.

#253 · Neurological Physiotherapy

What does the Alberta Infant Motor Scale (AIMS) measure and how does it guide decisions?

Answer

AIMS observes spontaneous movement in prone, supine, sitting and standing from birth to independent walking. It gives a percentile for age, helping identify delay and track change, but it does not diagnose the cause.

#254 · Neurological Physiotherapy

Why is tummy time with supported play a core home programme task for an infant with delayed motor development?

Answer

Prone play builds neck, trunk and shoulder extensor strength needed for rolling, crawling and sitting. Short, frequent, parent-led sessions while the infant is awake and supervised suit family routines and carry-over.

#255 · Neurological Physiotherapy

How should you obtain consent and involve the family when treating a baby with motor delay?

Answer

Seek informed consent from the parent or legal guardian, explain findings in plain language, and set goals with them. Use an interpreter if English is limited, and be culturally safe when working with Aboriginal and Torres Strait Islander families.

#256 · Neurological Physiotherapy

What are the key features of congenital muscular torticollis on assessment?

Answer

Head tilts to one side with chin rotation to the opposite side, often with a tight sternocleidomastoid, sometimes a palpable mass, and limited passive cervical rotation. Early stretching and positioning give the best outcomes.

#257 · Neurological Physiotherapy

Which findings suggest torticollis is not muscular and needs medical referral?

Answer

Neurological signs, vision or hearing concerns, a tilt that appears later in infancy, vomiting, or failure to improve with treatment. Non-muscular causes such as ocular, spinal or posterior fossa problems need urgent medical review.

#258 · Neurological Physiotherapy

What is a core home programme for an infant with congenital muscular torticollis?

Answer

Teach parents gentle passive stretching of the tight side, positioning and handling that encourage active rotation to the restricted side, and regular supervised tummy time. Consistent daily practice by parents drives progress.

#259 · Neurological Physiotherapy

How does positional plagiocephaly differ from craniosynostosis, and why does it matter?

Answer

Plagiocephaly gives a parallelogram head shape with ear shift and flattening from sustained positioning, while craniosynostosis is early suture fusion with a ridge and abnormal shape. Suspected synostosis needs prompt medical referral, not repositioning.

#260 · Neurological Physiotherapy

What safe-sleep advice must you keep consistent when advising repositioning for plagiocephaly?

Answer

Babies must still be placed on their backs to sleep, on a firm flat surface, in line with safe sleeping guidance. Repositioning strategies apply to awake, supervised time such as tummy time and varied carrying.

#261 · Neurological Physiotherapy

What sensory tests screen for peripheral neuropathy in a person with diabetes?

Answer

Test light touch with a 10 g monofilament, vibration with a 128 Hz tuning fork, pinprick and ankle reflexes at the feet. Loss of protective sensation means a high risk of unnoticed injury and ulceration.

#262 · Neurological Physiotherapy

Why is daily foot inspection taught to a person with diabetic peripheral neuropathy?

Answer

Loss of protective sensation means blisters, cuts and pressure areas go unnoticed and can become ulcers or infection. Daily checks, well-fitting footwear and never walking barefoot reduce ulcer and amputation risk.

#263 · Neurological Physiotherapy

What exercise programme suits a person with diabetic neuropathy and balance deficits?

Answer

Combine balance training, lower limb strengthening and regular aerobic activity, progressed gradually and supervised for safety. This improves function and falls risk, and exercise also supports glycaemic control.

#264 · Neurological Physiotherapy

Which sign in a person with diabetes and neuropathy needs same-day medical review?

Answer

A hot, red, swollen foot, an open wound, or signs of infection such as discharge, odour or spreading redness. These may indicate infection or Charcot foot, so offload the foot and escalate rather than continue exercise.

#265 · Neurological Physiotherapy

What precaution applies when exercising a person with diabetes who uses insulin?

Answer

Know their hypoglycaemia symptoms, check glucose before and after exercise where advised, and have fast-acting carbohydrate available. Do not exercise through symptoms of a hypo, and coordinate with their GP or diabetes educator.

#266 · Neurological Physiotherapy

What is the typical presentation of Charcot-Marie-Tooth disease?

Answer

A slowly progressive, inherited, length-dependent neuropathy causing distal weakness and wasting, foot drop, pes cavus, hammer toes and distal sensory loss, usually beginning in adolescence or early adulthood with a family history.

#267 · Neurological Physiotherapy

Why must resistance exercise be moderate in Charcot-Marie-Tooth disease?

Answer

Weak, denervated muscles fatigue easily and may be damaged by heavy overwork. Use submaximal strengthening with rest, avoid exercising to exhaustion, and monitor for delayed soreness or weakness after sessions.

#268 · Neurological Physiotherapy

How can an ankle-foot orthosis help a person with Charcot-Marie-Tooth disease and foot drop?

Answer

An AFO holds the ankle in a neutral position during swing, reducing tripping and improving gait efficiency and safety. Refer to an orthotist for fitting, and review regularly as the condition progresses.

#269 · Neurological Physiotherapy

What is a key preventive priority for contractures and deformity in Charcot-Marie-Tooth disease?

Answer

Maintain ankle and toe range with regular stretching of the calf and plantar structures, supportive footwear and orthoses. Preventing fixed deformity preserves walking ability and reduces pain and falls.

#270 · Neurological Physiotherapy

What genetic counselling issue should you raise sensitively with a person who has Charcot-Marie-Tooth disease?

Answer

The condition is usually inherited, so family members may be affected. Recommend referral to a genetics service through their GP rather than giving inheritance risks yourself, and respect their privacy and choices.

#271 · Neurological Physiotherapy

What are the typical symptoms of peripheral vestibular hypofunction?

Answer

Dizziness or vertigo with head movement, blurred or bouncing vision during head movement (oscillopsia), and imbalance, especially in the dark or on uneven surfaces. Hearing is often normal unless the inner ear is also affected.

#272 · Neurological Physiotherapy

What is the purpose of gaze stabilisation exercises in vestibular rehabilitation?

Answer

Repeated head movements while keeping eyes fixed on a target drive adaptation of the vestibulo-ocular reflex. They reduce blurred vision and dizziness, and need frequent daily practice, progressed as symptoms allow.

#273 · Neurological Physiotherapy

Which findings with dizziness are red flags for a central cause requiring urgent medical referral?

Answer

Sudden severe headache, double vision, slurred speech, facial or limb weakness, new neurological signs, or vertical nystagmus. These suggest stroke or other central pathology, so stop treatment and call for emergency medical help.

#274 · Neurological Physiotherapy

What does the Dizziness Handicap Inventory measure and how is it used?

Answer

A self-report questionnaire of the physical, emotional and functional effects of dizziness. Use it at baseline and to track change with rehabilitation, alongside balance tests, to show progress and guide goals.

#275 · Neurological Physiotherapy

How do you document and manage falls risk for a person with vestibular hypofunction living in the community?

Answer

Assess balance and gait, review home hazards, advise on lighting, footwear and avoiding sudden head turns on stairs, and record the plan. Share the risk and advice with the GP, and refer to an occupational therapist if needed.

#276 · Neurological Physiotherapy

Which positional test confirms posterior canal BPPV, and what finding is diagnostic?

Answer

The Dix-Hallpike test. Diagnostic findings are a brief latency, then vertigo with torsional (upbeating geotropic) nystagmus towards the affected ear that fatigues within about a minute. Absence of nystagmus does not fully exclude BPPV.

#277 · Neurological Physiotherapy

What is the treatment of choice for posterior canal BPPV, and why does it work?

Answer

The Epley canalith repositioning manoeuvre. It uses sequenced head positions so gravity moves displaced otoconia out of the semicircular canal into the vestibule, where they no longer trigger vertigo.

#278 · Neurological Physiotherapy

A patient with vertigo also has new double vision, slurred speech and facial numbness. What do you do?

Answer

Stop the vestibular testing and arrange urgent medical assessment, for example via the emergency department. These central signs suggest a brainstem or cerebellar stroke, not BPPV, and positional manoeuvres would be unsafe.

#279 · Neurological Physiotherapy

Before Dix-Hallpike testing in an older adult, which screening check reduces risk?

Answer

Check for neck instability, severe cervical stenosis, carotid or vertebrobasilar concerns, and ability to tolerate neck extension and rotation. If unsafe, use a modified technique such as the side-lying test or refer for medical review.

#280 · Neurological Physiotherapy

What should you explain and obtain before performing a provocative positional manoeuvre on a patient with vertigo?

Answer

Explain that the test will deliberately bring on dizziness and nausea, what position changes occur, and that they can stop at any time. Obtain informed consent and have a bucket and a safe plinth setup ready.

#281 · Neurological Physiotherapy

Why is a mobility decline in a person with dementia not simply attributed to the dementia itself?

Answer

Acute change may reflect delirium, infection, pain, constipation, medication effects or dehydration. Screen for reversible causes and liaise with the GP or ward team before assuming progression of the underlying dementia.

#282 · Neurological Physiotherapy

What is the evidence-based core of falls prevention exercise for community-dwelling older adults, including those with mild dementia?

Answer

Regular, progressive and challenging balance and strength training, totalling at least 3 hours a week, reduces falls. Programmes are tailored to cognition, with simple, repeated instructions.

#283 · Neurological Physiotherapy

How do you adapt communication when treating a person with moderate dementia?

Answer

Use short single-step instructions, demonstration and gesture, a calm familiar environment, consistent staff and routine, and carer involvement. Avoid correcting or arguing, and reassess understanding rather than assuming it.

#284 · Neurological Physiotherapy

Which outcome measures are commonly used to track mobility and falls risk in older people with cognitive impairment?

Answer

Timed Up and Go, gait speed over a short distance, the Berg Balance Scale and the 30-second sit-to-stand. Each is quick and repeatable, showing change over time and guiding the intensity and supervision level of therapy.

#285 · Neurological Physiotherapy

A person with dementia lacks capacity to consent to a physiotherapy programme. What is the correct approach?

Answer

Capacity is decision-specific. Assess it, support decision-making where possible, and if lacking, involve the appointed substitute decision-maker under state law. Document the process and proceed only with treatment in their best interests.

#286 · Neurological Physiotherapy

What distinguishes delirium from dementia at the bedside?

Answer

Delirium has acute onset, fluctuating course, inattention and often altered arousal, over hours to days. Dementia is gradual and progressive with stable alertness. Delirium is a medical emergency needing the team to find the cause.

#287 · Neurological Physiotherapy

Name a tool commonly used on Australian wards to screen for delirium.

Answer

The 4AT or the Confusion Assessment Method (CAM). Both screen for acute change and inattention. A positive result should be escalated to the medical team rather than managed by physiotherapy alone.

#288 · Neurological Physiotherapy

How can physiotherapy help prevent or reduce delirium in an acute ward patient?

Answer

Early mobilisation, getting patients out of bed and walking, supports orientation, sleep-wake cycle and function. Combine with glasses, hearing aids, hydration, daylight and avoiding restraints, as part of the multidisciplinary approach.

#289 · Neurological Physiotherapy

A patient with hyperactive delirium is agitated and pulling at lines. What is your priority during a physiotherapy session?

Answer

Safety first: do not force treatment, keep the environment calm, reduce stimulation, involve nursing staff and escalate to the team. Defer non-urgent exercise and document the behaviour and your decision.

#290 · Neurological Physiotherapy

When can a ward patient with delirium be treated without their consent?

Answer

Only where capacity is lacking and the treatment is necessary and in their best interests, following hospital policy and any substitute decision-maker involvement. Use the least restrictive approach and document the reasoning clearly.

#291 · Neurological Physiotherapy

Which examination findings suggest cervical myelopathy rather than a simple radiculopathy?

Answer

Upper motor neuron signs: hyperreflexia, positive Hoffmann sign, clonus, upgoing plantar response, spastic gait and clumsy hands. Radiculopathy shows lower motor neuron signs in a dermatomal or myotomal pattern.

#292 · Neurological Physiotherapy

What is a red-flag symptom pattern that should make you suspect cervical myelopathy?

Answer

Progressive loss of hand dexterity (dropping things, difficulty with buttons), unsteady gait, and bilateral hand numbness, sometimes with bladder disturbance. Arrange prompt medical referral for imaging and surgical opinion.

#293 · Neurological Physiotherapy

Why should cervical manipulation or aggressive end-range mobilisation be avoided in suspected myelopathy?

Answer

Stress on a compromised spinal cord can worsen neurological injury. Avoid high-velocity thrust and end-range loading, and refer promptly. Gentle, non-provocative activity may be acceptable while awaiting medical review.

#294 · Neurological Physiotherapy

What does the Hoffmann sign test, and how is it performed?

Answer

It tests for upper motor neuron involvement. Flick the distal phalanx of the middle finger; thumb and index finger flexion in response is a positive sign, suggesting corticospinal tract compromise, for example in myelopathy.

#295 · Neurological Physiotherapy

What changes in function should you monitor and report in a patient with known degenerative cervical myelopathy awaiting surgery?

Answer

Worsening gait, hand function, balance or new bowel and bladder disturbance. Any deterioration should trigger urgent notification of the surgeon or GP, as delay can mean permanent neurological loss.

#296 · Neurological Physiotherapy

Which test is used to screen for median nerve involvement at the carpal tunnel and what does it provoke?

Answer

The Phalen test or the carpal compression test reproduces numbness and tingling in the median nerve distribution. Positive tests support carpal tunnel syndrome, though they are best combined with history and other findings.

#297 · Neurological Physiotherapy

What is the purpose of neurodynamic tests such as the upper limb tension test?

Answer

They lengthen or load a nerve to see whether symptoms reproduce, and whether they change with distal or proximal structural differentiation, such as ankle or neck movement. A positive result suggests mechanosensitive neural tissue.

#298 · Neurological Physiotherapy

How are neural mobilisation techniques safely progressed for a peripheral nerve entrapment?

Answer

Start with gentle sliders that move the nerve with minimal tension, and progress to tensioners only if symptoms stay mild and settle quickly. Avoid aggravating symptoms, and monitor for worsening numbness or weakness.

#299 · Neurological Physiotherapy

A patient with suspected nerve entrapment develops progressive wasting and weakness of the thenar muscles. What does this mean for management?

Answer

Muscle wasting and progressive motor loss suggest significant axonal compromise and warrant prompt referral for nerve conduction studies and surgical opinion. Conservative care alone is no longer appropriate.

#300 · Neurological Physiotherapy

What advice on activity modification helps a worker with a suspected compensable nerve entrapment injury?

Answer

Reduce sustained postures and repetitive tasks, introduce micro-breaks, adjust workstation setup, and liaise with the employer and insurer. Document restrictions and progress, and follow the relevant workers' compensation scheme requirements.

#301 · Neurological Physiotherapy

A newborn has an Erb's palsy pattern after a difficult delivery. Which arm posture is typical, and which nerve roots are involved?

Answer

The arm hangs adducted and internally rotated with the elbow extended and forearm pronated (waiter's tip), from C5 and C6 injury. Hand grasp is usually preserved, which helps distinguish it from a total plexus injury.

#302 · Neurological Physiotherapy

After a motorcycle crash, an adult has a flail arm, absent sensation and a Horner's sign on the same side. What does the Horner's sign suggest?

Answer

Horner's sign suggests a preganglionic injury at C8 to T1 with sympathetic chain involvement, which carries a poorer prognosis for spontaneous recovery. Report it promptly to the medical team, as early surgical review may be needed.

#303 · Neurological Physiotherapy

Why is passive range of motion at the shoulder, elbow and wrist prioritised while waiting for nerve recovery after brachial plexus injury?

Answer

Joints and soft tissue stiffen and contract while muscles are denervated. Gentle, full-range passive movement preserves joint mobility so the limb is ready to use if reinnervation occurs. Avoid forceful stretching of the shoulder, which may be subluxed.

#304 · Neurological Physiotherapy

How should a flaccid, partly denervated arm be protected from harm in a patient with a brachial plexus injury and reduced sensation?

Answer

Support the arm in a sling or on pillows to prevent traction on the shoulder, and teach daily skin and joint checks. Loss of sensation means burns, pressure injury and trauma go unnoticed, so education on visual monitoring is essential.

#305 · Neurological Physiotherapy

A patient with a brachial plexus injury cannot decide on nerve transfer surgery and asks you to choose for them. What is the appropriate response?

Answer

Explain that the decision belongs to the patient, supported by the surgeon, and that timing matters for outcomes. Provide balanced information, encourage questions to the treating team, and document the discussion. Consent for surgery is obtained by the surgeon, not the physiotherapist.

#306 · Neurological Physiotherapy

What is the purpose of serial casting for a spastic ankle plantarflexor contracture?

Answer

Casting applies a sustained low-load stretch to lengthen the muscle-tendon unit and improve dorsiflexion range. It is often combined with botulinum toxin to reduce the muscle's resistance, followed by stretching, strengthening and splinting to keep the gain.

#307 · Neurological Physiotherapy

How long after a botulinum toxin injection does the effect typically begin, and when should physiotherapy be timed around it?

Answer

Effect usually starts within several days and peaks at about two to four weeks. Use this window for intensive stretching, strengthening and task practice, as the reduced tone allows better movement and motor learning. Effects wear off over about three months.

#308 · Neurological Physiotherapy

What is the difference between spasticity and a fixed contracture, and why does it change your treatment plan?

Answer

Spasticity is velocity-dependent resistance to passive stretch from neural overactivity; a contracture is a fixed shortening of muscle and soft tissue. Spasticity may respond to toxin and stretching, whereas an established contracture needs casting, splinting or surgical review.

#309 · Neurological Physiotherapy

Which outcome measure grades the resistance felt during passive movement of a spastic joint, and what is its main limitation?

Answer

The Modified Ashworth Scale grades resistance to passive stretch from 0 to 4. It is quick but has limited reliability and does not separate neural from soft tissue contributions, so combine it with range of motion and functional measures.

#310 · Neurological Physiotherapy

A client with spasticity is cast and complains of burning and numbness at the heel. What do you do?

Answer

Remove the cast promptly and check the skin and circulation. Pain, numbness, colour change or pressure marks signal possible pressure injury or nerve compromise. Document, report to the prescriber and review padding and fit before any recasting.

#311 · Neurological Physiotherapy

In a patient with a spinal cord injury at T6 or above, a sudden pounding headache, flushing above the lesion and BP of 190/110 mmHg occur. What is the emergency?

Answer

This is autonomic dysreflexia, a medical emergency. Sit the person upright, loosen tight clothing, call for urgent help and look for the trigger, commonly a blocked catheter, full bladder or bowel. Do not leave them alone.

#312 · Neurological Physiotherapy

Which bladder problem is expected after an upper motor neuron spinal cord lesion, and what does it mean for physiotherapy?

Answer

A spastic or reflex bladder with poor coordination between the detrusor and sphincter, which risks high pressures and reflux. Physiotherapists should know the bladder regimen, watch for triggers such as full bladder that increase spasms, and refer concerns to the continence nurse or urologist.

#313 · Neurological Physiotherapy

Why is a regular pressure-relief routine taught to wheelchair users with spinal cord injury?

Answer

Sustained pressure over bony points such as the ischial tuberosities causes tissue ischaemia and pressure injuries, made worse by absent sensation and moisture. Teach regular weight shifts or push-ups, cushion checks and daily skin inspection.

#314 · Neurological Physiotherapy

Why should you check the bladder status before a long standing or transfer session in a client with neurogenic bladder?

Answer

A full bladder can provoke spasticity, autonomic dysreflexia in high lesions, and leakage that affects dignity and skin. Confirm the client's emptying routine, catheter position and bag drainage, and keep tubing free of kinks during transfers.

#315 · Neurological Physiotherapy

What bowel care principle supports a client with a neurogenic bowel who is starting an upright, active rehabilitation program?

Answer

A predictable bowel routine, usually timed after meals with diet, fluids and aperients as prescribed, reduces accidents. Activity and upright positioning help gut motility. Coordinate session times with the routine and refer problems to the nurse or doctor.

#316 · Neurological Physiotherapy

A person with epilepsy has a generalised tonic-clonic seizure during a gym session. What is the correct first aid?

Answer

Note the time, protect the head, clear hazards, do not restrain or place anything in the mouth, and roll them onto their side once movements stop. Call an ambulance if it lasts over five minutes, repeats, or injury or breathing difficulty occurs.

#317 · Neurological Physiotherapy

After a seizure ends, what monitoring does a physiotherapist provide while waiting for recovery?

Answer

Stay with the person, keep them on their side, check breathing and responsiveness, and reassure them as confusion is common. Allow rest and do not offer food or drink until fully alert. Document the event and notify the treating team.

#318 · Neurological Physiotherapy

What exercise advice is appropriate for a person with well-controlled epilepsy?

Answer

Regular exercise is encouraged, as it supports health and may not increase seizure risk. Advise supervision or precautions for water-based or heights activities, adequate sleep, hydration and medication adherence, individualised to seizure type and control.

#319 · Neurological Physiotherapy

A client with epilepsy mentions they missed their anti-seizure medication for several days. How do you respond?

Answer

Encourage them to contact their doctor promptly, as missed doses raise seizure risk. Avoid high-risk activities until reviewed, record what was disclosed, and stay within scope by not advising on medication changes.

#320 · Neurological Physiotherapy

A patient discloses a seizure that occurred while driving. What is the professional approach?

Answer

Advise them to seek medical review and follow state driver licensing requirements, which differ by jurisdiction. Document your advice and discussion, respect privacy, and escalate through the treating doctor rather than reporting independently unless a legal duty applies.

#321 · Neurological Physiotherapy

A person with post-polio syndrome reports new weakness and fatigue years after childhood polio. Which exercise principle applies?

Answer

Use submaximal, low-intensity exercise with rest breaks and energy conservation, as overuse of already-compromised motor units can worsen weakness and fatigue. Avoid heavy, fatiguing resistance training and monitor symptoms over the next day.

#322 · Neurological Physiotherapy

How does the pacing approach help a client with post-polio syndrome manage daily fatigue?

Answer

Break tasks into short activities with planned rests, prioritise important tasks and spread them across the day. This reduces overexertion that triggers a delayed fatigue crash. Review the plan regularly and set goals around function, not maximal effort.

#323 · Neurological Physiotherapy

Which assistive measures may reduce energy cost and protect weak muscles in a person with post-polio syndrome?

Answer

Orthoses, mobility aids or a scooter for longer distances can reduce overload on weakened limbs and joints. Trial equipment, teach correct use and review fit, with referral to an orthotist or occupational therapist as needed.

#324 · Neurological Physiotherapy

A client with post-polio syndrome has new swallowing and breathing difficulty. What should you do?

Answer

Refer promptly to the GP or specialist, as respiratory muscle weakness and sleep-disordered breathing can occur and need medical assessment. Report symptoms such as morning headaches or breathlessness, and avoid heavy respiratory loading until reviewed.

#325 · Neurological Physiotherapy

An older man with post-polio syndrome from a regional area struggles to attend clinic. How might service delivery be adapted?

Answer

Consider telehealth for education and program review, with local support for hands-on needs, and funding options such as Medicare chronic disease items or My Aged Care. Obtain informed consent for telehealth and check privacy and technology access.

#326 · Neurological Physiotherapy

A client with a frontal lobe injury is impulsive and ignores your safety instructions during gait practice. How should you adapt the session?

Answer

Use short, consistent, one-step cues, a structured routine and close supervision, and remove hazards. Impaired judgement and impulsivity raise falls risk, so safety strategies cannot rely on the client remembering or self-monitoring.

#327 · Neurological Physiotherapy

Why should you screen for depression in a client who is not engaging with stroke rehabilitation?

Answer

Post-stroke depression is common and reduces participation, motivation and outcomes. Raise concerns with the treating doctor or GP for formal assessment and referral, and adjust goals and session structure meanwhile, rather than labelling the client non-compliant.

#328 · Neurological Physiotherapy

A client with moderate cognitive impairment after traumatic brain injury cannot retain new exercises. What learning approach suits them best?

Answer

Errorless learning with consistent routines, task-specific practice in the real environment and carer involvement. Procedural memory is often better preserved than declarative memory, so repetition of the same task suits better than verbal instruction.

#329 · Neurological Physiotherapy

A client with severe cognitive impairment cannot understand the proposed treatment. What must you do about consent?

Answer

Assess decision-making capacity for this specific decision, support the client to understand where possible, and if capacity is lacking seek consent from the legally appointed substitute decision-maker under state or territory law. Document the process.

#330 · Neurological Physiotherapy

What does unilateral spatial neglect after right hemisphere stroke look like, and why does it matter for therapy?

Answer

The client fails to attend to or respond to the left side of space despite intact sensation and vision. It raises falls and collision risk and limits function, so cue attention to the neglected side and arrange the environment safely.

#331 · Neurological Physiotherapy

How do you choose between a performance-based and a self-report outcome measure for a neurological client?

Answer

Match the measure to the question: performance-based tests (such as 10 metre walk test) record what the person does in the clinic, while self-report captures perceived function and participation. Using both gives a fuller picture and guides goals.

#332 · Neurological Physiotherapy

What does the Berg Balance Scale measure and how does it guide decisions?

Answer

It scores performance of 14 functional balance tasks, with lower scores indicating greater balance impairment and falls risk. Use it to identify balance deficits, set goals and track change, not as the only predictor of falls.

#333 · Neurological Physiotherapy

Why use the 6 Minute Walk Test with a client after stroke who walks household distances?

Answer

It measures walking endurance and community ambulation capacity over a standardised time. It guides goal setting for community walking and cardiorespiratory fitness, and requires monitoring of symptoms, heart rate and rests during the test.

#334 · Neurological Physiotherapy

What is a floor effect in an outcome measure, and why does it matter for severely affected neurological clients?

Answer

A floor effect means many people with severe impairment score at or near the minimum, so the tool cannot detect small improvements. Choose a more sensitive measure suited to their level, such as the Trunk Impairment Scale or a sitting balance test.

#335 · Neurological Physiotherapy

How do you interpret change on an outcome measure when deciding whether treatment worked?

Answer

Compare the change with the measure's minimal detectable change, not just the raw difference. Change smaller than measurement error may not be real, while change beyond it, ideally also reaching the minimal clinically important difference, supports a genuine response.

#336 · Neurological Physiotherapy

A transgender man with spinal cord injury attends for rehabilitation. How should you record his name and pronouns?

Answer

Ask him how he wishes to be addressed and which pronouns to use, then use them consistently in conversation and documentation. Share information only with his consent, as health records are covered by privacy law and trust affects engagement.

#337 · Neurological Physiotherapy

A gender diverse client with multiple sclerosis feels unsafe in a shared gym. What should you consider?

Answer

Offer choices such as private spaces, quieter session times and a say in who is present, and use respectful, non-assumptive language. Feeling safe affects attendance and participation, and discrimination is both unprofessional and unlawful.

#338 · Neurological Physiotherapy

A client on gender-affirming hormone therapy has had a stroke. How does this affect your physiotherapy assessment?

Answer

Document it as part of their medical history and inform the treating team, as hormone therapy may influence clotting, bone density and cardiovascular risk. Do not alter exercise safety limits yourself; seek medical advice about concerns.

#339 · Neurological Physiotherapy

When examining the body of a gender diverse client with neurological impairment, what is the approach to consent?

Answer

Explain the purpose and extent of each examination, seek permission before touch, offer a chaperone or support person, and allow the client to decline or pause. Clear, continuing consent respects dignity and reduces distress.

#340 · Neurological Physiotherapy

What is the purpose of body weight support treadmill training for a person with incomplete spinal cord injury?

Answer

It unloads part of body weight so the person can practise stepping with a safer, more normal pattern and higher repetition. It supports task-specific, intensive gait practice, with progression by reducing support as ability improves.

#341 · Neurological Physiotherapy

What must you check before starting robotic-assisted gait training with a client?

Answer

Check medical stability, skin integrity, joint range, bone health, spasticity, blood pressure response and cognitive ability to follow instructions, plus device suitability for height and weight. Contraindications and precautions must be cleared with the team.

#342 · Neurological Physiotherapy

A client with orthostatic hypotension is being stood for body weight support gait training. What precaution applies?

Answer

Monitor blood pressure and symptoms such as dizziness and pallor, progress upright positions gradually, use abdominal binders or stockings if advised, and be ready to lower the client. Low blood pressure can cause fainting and falls.

#343 · Neurological Physiotherapy

Why is robotic gait training not a replacement for overground walking practice?

Answer

Evidence suggests robotics works best as an addition to usual care, not instead of it. Overground practice is needed for balance, variable terrain and real-life transfer, so combine the two according to goals and stage of recovery.

#344 · Neurological Physiotherapy

How do you make sure the client can use a robotic device in a safe and informed way?

Answer

Explain what the device does, risks, expected sensations and how to stop, obtain informed consent, and ensure trained staff supervise. Document consent and any adverse events, and report incidents through the service's incident system.

#345 · Neurological Physiotherapy

A child with hydrocephalus and a shunt has new vomiting, lethargy and a severe headache. What should you do?

Answer

Treat it as a possible shunt blockage or malfunction: stop therapy, monitor the child and arrange urgent medical review. This is a neurosurgical emergency because raised intracranial pressure can deteriorate quickly.

#346 · Neurological Physiotherapy

What are the warning signs of shunt malfunction in an infant with hydrocephalus?

Answer

Look for a bulging fontanelle, increasing head circumference, irritability, poor feeding, vomiting, sleepiness and downward eye deviation (sunsetting eyes). Report these promptly for urgent medical assessment and do not wait for the next review.

#347 · Neurological Physiotherapy

What manual handling precaution applies over the shunt valve and tubing site during treatment?

Answer

Avoid direct pressure, firm manipulation or prolonged lying on the valve and tubing track, and position gently. Pressure can damage the system or cause skin breakdown, so follow the neurosurgical team's specific advice for the child.

#348 · Neurological Physiotherapy

Why is head control practice important for a child with hydrocephalus, and how should you support it?

Answer

A large head and low tone can delay head control and motor milestones. Use supported prone, sitting and play positions to build strength and symmetry, grade activity to tolerance and watch for signs of raised intracranial pressure.

#349 · Neurological Physiotherapy

A child with hydrocephalus uses an interpreter-needing family's language at home. How do you work with the parents?

Answer

Book a qualified interpreter (not a child or relative), explain shunt warning signs clearly with written or visual aids, and confirm understanding through teach-back. Parents are the first to spot deterioration, so accurate communication is a safety issue.

#350 · Neurological Physiotherapy

Which findings point to an upper motor neurone lesion rather than a lower motor neurone lesion?

Answer

Raised tone with spasticity, brisk reflexes, an extensor plantar response and clonus suggest an upper motor neurone lesion. Weakness with low tone, absent reflexes, wasting and fasciculations suggest a lower motor neurone lesion.

#351 · Cardiorespiratory Physiotherapy

A patient with community acquired pneumonia has a productive cough, coarse crackles and fever. When is airway clearance most clearly indicated?

Answer

When there is sputum retention causing difficulty clearing secretions, shown by audible or palpable secretions and coarse crackles. Physiotherapy does not treat pneumonia consolidation itself, so a dry, non-productive patient gains little from airway clearance techniques.

#352 · Cardiorespiratory Physiotherapy

Which positioning strategy helps ventilation-perfusion matching in unilateral lobar pneumonia?

Answer

Place the healthy lung dependent (good lung down) so the better-perfused lung is also the better-ventilated one, improving V/Q matching and oxygenation. If the aim is secretion drainage, place the affected lung uppermost. Choose by the goal and monitor SpO2.

#353 · Cardiorespiratory Physiotherapy

What is the purpose of the active cycle of breathing technique in a patient with sputum retention?

Answer

It combines breathing control, thoracic expansion exercises and forced expiration (huff) to mobilise and clear secretions with less fatigue and less bronchospasm than repeated coughing. It needs the patient to be alert and able to follow instructions.

#354 · Cardiorespiratory Physiotherapy

A pneumonia patient reports sudden sharp pleuritic chest pain and new breathlessness on the ward. What should the physiotherapist consider?

Answer

Possible pleural effusion, pneumothorax or pulmonary embolism. Pause treatment, check vital signs and SpO2, and escalate promptly to the medical team. Pleuritic pain limits deep breathing, so do not simply continue the planned chest techniques.

#355 · Cardiorespiratory Physiotherapy

You are using an interpreter for a pneumonia patient who speaks limited English. How do you teach huffing and coughing technique?

Answer

Use an accredited interpreter (for example via TIS National or hospital service) rather than family, demonstrate the technique, then use teach-back to confirm understanding. Family interpreters risk errors and compromise confidentiality and consent.

#356 · Cardiorespiratory Physiotherapy

Why are patients with aspiration pneumonia at higher risk of recurrence, and what is the physiotherapist's role in prevention?

Answer

Ongoing dysphagia, reduced consciousness or poor oral hygiene allows repeated aspiration. Physiotherapists position upright for meals and activity, liaise with speech pathology for swallow assessment, and report suspected aspiration rather than assume swallow safety.

#357 · Cardiorespiratory Physiotherapy

Which lung segments are most commonly affected by aspiration in an upright patient versus a supine patient?

Answer

Upright aspiration tends to affect the basal segments of the lower lobes, especially the right. Supine aspiration tends to affect the posterior segment of the right upper lobe and the superior segment of the right lower lobe. This guides positioning for drainage.

#358 · Cardiorespiratory Physiotherapy

A ventilated patient has hospital acquired pneumonia. Which physiotherapy measure helps reduce further ventilator-associated complications?

Answer

Early mobilisation and sitting upright (head of bed raised, unless contraindicated) reduce secretion pooling and aspiration risk. Assess haemodynamic stability, sedation level and oxygen requirements first, and work with the ICU team on safe progression.

#359 · Cardiorespiratory Physiotherapy

When should manual techniques such as percussion or vibration be avoided or modified in a pneumonia patient?

Answer

Avoid or modify with rib fractures, unstable cardiovascular state, bleeding risk or coagulopathy, severe osteoporosis, or marked haemoptysis. Choose gentler options like positioning and active techniques. Evidence for routine percussion in pneumonia is weak, so reserve it for clear sputum retention.

#360 · Cardiorespiratory Physiotherapy

What baseline observations must you check before and during treatment of a hospital acquired pneumonia patient?

Answer

Heart rate, respiratory rate, blood pressure, SpO2, temperature, work of breathing and conscious state, plus oxygen delivery device. Compare with the medical emergency response triggers, and stop and escalate if the patient deteriorates during treatment.

#361 · Cardiorespiratory Physiotherapy

What are the key postoperative pulmonary complications after thoracic surgery that physiotherapy aims to prevent?

Answer

Atelectasis, sputum retention, pneumonia and respiratory failure, driven by pain, reduced lung volumes and impaired cough. Early mobilisation, deep breathing, adequate analgesia and supported coughing form the core of prevention.

#362 · Cardiorespiratory Physiotherapy

How should you manage a chest drain during mobilisation of a postoperative patient?

Answer

Keep the collection system below chest level and upright, avoid kinking or tension on tubing, and never clamp the drain unless instructed. Carry or hang the bottle securely so the underwater seal is maintained, and observe for air leak or drainage change.

#363 · Cardiorespiratory Physiotherapy

You see continuous bubbling in the underwater seal chamber during a patient's cough. What does this suggest and what do you do?

Answer

Bubbling on coughing is expected with a pneumothorax, but continuous bubbling suggests an ongoing air leak or loose connection. Check connections, note the finding and inform the medical or nursing team. Do not clamp the tube.

#364 · Cardiorespiratory Physiotherapy

A chest drain accidentally disconnects from the tubing during treatment. What is the immediate action?

Answer

Reconnect immediately if the tubing is clean and the seal can be restored, or place the tube end in sterile water as a temporary seal, and call for help. Do not clamp a drain with an air leak because it risks tension pneumothorax.

#365 · Cardiorespiratory Physiotherapy

Why is shoulder range of movement addressed early after a thoracotomy?

Answer

The surgical approach divides muscles and causes pain, leading to guarding and a stiff ipsilateral shoulder. Early active-assisted range and posture work prevent loss of shoulder movement and improve breathing mechanics and function, within surgeon precautions.

#366 · Cardiorespiratory Physiotherapy

How do you support coughing and huffing after thoracic surgery?

Answer

Give wound support with a folded towel, pillow or hands over the incision and ensure adequate analgesia first. Splinting reduces pain and allows effective cough. Pain limiting cough is the main barrier, so liaise with the team about analgesia timing.

#367 · Cardiorespiratory Physiotherapy

A patient is two days post lobectomy with a new surgical emphysema area on the neck. How do you respond?

Answer

Mark the margins, record the extent, and inform the medical team, as subcutaneous emphysema may indicate an air leak or drain malfunction. Monitor for spread, breathing difficulty or airway compromise, and escalate urgently if it progresses.

#368 · Cardiorespiratory Physiotherapy

A nursing assistant has been asked to walk a post-thoracic surgery patient with a chest drain. What must you consider before delegating?

Answer

Delegate only when the patient is stable, the task is within the assistant's training and a clear plan exists, with supervision available. You remain accountable for the assessment, plan and outcome. New drain concerns or deterioration should be escalated back to you.

#369 · Cardiorespiratory Physiotherapy

What are the typical signs of a flail segment on assessment?

Answer

Paradoxical movement of a chest wall segment (it moves inward on inspiration and outward on expiration) with pain, bruising and tenderness over multiple adjacent rib fractures, each broken in two or more places. It impairs ventilation and often underlies contusion.

#370 · Cardiorespiratory Physiotherapy

Why is pain control central to physiotherapy management of rib fractures?

Answer

Pain causes shallow breathing, poor cough, sputum retention and pneumonia, especially in older people. Regional analgesia such as epidural or nerve blocks allows deep breathing and mobilisation, so coordinate treatment around analgesia and report uncontrolled pain.

#371 · Cardiorespiratory Physiotherapy

A patient with rib fractures develops increasing breathlessness, falling SpO2 and tracheal deviation. What are you concerned about?

Answer

Tension pneumothorax or haemothorax, which is a medical emergency. Stop treatment, call the emergency response, give oxygen as per protocol and monitor. Rib fractures can lacerate the lung or vessels, so delayed deterioration is possible.

#372 · Cardiorespiratory Physiotherapy

Which techniques are generally avoided in the acute phase of flail chest?

Answer

Vigorous percussion, vibration over the injured area, and unsupported forced coughing, because they risk further injury and pain. Use positioning, supported huffing, incentive spirometry and early mobilisation as tolerated, per medical advice and analgesia.

#373 · Cardiorespiratory Physiotherapy

What are the key features of a severe acute asthma exacerbation that need urgent medical escalation?

Answer

Inability to speak in full sentences, accessory muscle use, silent chest, exhaustion, confusion, cyanosis or SpO2 below target despite oxygen. These indicate life-threatening asthma. Stop physiotherapy, call for urgent medical help and follow the hospital emergency response.

#374 · Cardiorespiratory Physiotherapy

What is the role of peak expiratory flow in assessing and monitoring asthma?

Answer

It gives a simple measure of airflow limitation and response to treatment when compared with the patient's personal best or predicted value. A falling reading indicates worsening control. It supports a written asthma action plan, but must not replace clinical assessment.

#375 · Cardiorespiratory Physiotherapy

A child with asthma wants to return to sport after an exacerbation. What advice should the physiotherapist give?

Answer

Follow the written asthma action plan, use reliever medication as prescribed before exercise if advised, warm up gradually and avoid known triggers. Seek GP review if symptoms are persistent. Graded return and regular review support safe participation.

#376 · Cardiorespiratory Physiotherapy

Which blood gas finding in a COPD exacerbation suggests a patient may need non-invasive ventilation?

Answer

Acute hypercapnic respiratory acidosis, a raised PaCO2 with a low pH despite optimal medical therapy, is the classic indication. Tell the medical team promptly, as starting NIV is a medical decision.

#377 · Cardiorespiratory Physiotherapy

How does non-invasive ventilation help a patient in a COPD exacerbation?

Answer

It supports alveolar ventilation and unloads tiring respiratory muscles, lowering work of breathing and CO2 retention, and can avoid intubation. Physiotherapists help with positioning, mask fit, comfort and monitoring the response.

#378 · Cardiorespiratory Physiotherapy

A patient on NIV becomes drowsy and confused with worsening breathing. What should you do?

Answer

Treat it as possible rising CO2 or NIV failure. Stay with the patient, keep the airway safe, and escalate urgently to the medical team or rapid response rather than continuing your treatment.

#379 · Cardiorespiratory Physiotherapy

Which positions ease breathlessness for a patient in a COPD exacerbation?

Answer

Supported forward lean sitting or high side lying fixes the shoulder girdle so accessory muscles work efficiently, easing breathlessness. Pair with pursed lip breathing and a calm, paced approach.

#380 · Cardiorespiratory Physiotherapy

A patient admitted with a COPD exacerbation refuses chest physiotherapy. How do you proceed?

Answer

An adult with capacity may refuse. Explain benefits and risks in plain language, confirm understanding, respect the decision, document the discussion and refusal, and inform the treating team.

#381 · Cardiorespiratory Physiotherapy

What oxygen saturation target is usually set for a COPD patient at risk of hypercapnia?

Answer

A lower range, commonly 88 to 92 percent, unless the medical team specifies otherwise. Too much oxygen can worsen CO2 retention, so follow the documented target rather than titrating to 100 percent.

#382 · Cardiorespiratory Physiotherapy

What is the purpose of long term oxygen therapy in COPD?

Answer

It treats chronic hypoxaemia at rest and is used for most hours of the day, including sleep, to reduce strain on the heart and improve survival. A medical specialist prescribes it after stable-state assessment.

#383 · Cardiorespiratory Physiotherapy

What safety advice is essential for a patient using home oxygen?

Answer

No smoking, naked flames or heat sources near oxygen, keep equipment away from cooking, store cylinders upright and secured, and avoid oil based products on the face. Fire is the main hazard.

#384 · Cardiorespiratory Physiotherapy

A patient on home oxygen desaturates during exercise. How do you manage this?

Answer

Stop or slow, rest in a supported position with pursed lip breathing, and check recovery. Oxygen use with exercise is guided by the prescriber, so report repeated desaturation for review of oxygen needs.

#385 · Cardiorespiratory Physiotherapy

A patient on home oxygen who needs an interpreter is unsure how equipment support works. What is your role?

Answer

Book a qualified interpreter, not a family member. Funding and supply of home oxygen differ between states and territories, so refer to the treating doctor, social worker or local program rather than guessing eligibility.

#386 · Cardiorespiratory Physiotherapy

Why is airway clearance a daily priority in cystic fibrosis?

Answer

A faulty chloride channel makes mucus thick and sticky, which blocks airways and harbours infection. Regular clearance reduces mucus retention, helps preserve lung function and slows the development of bronchiectasis.

#387 · Cardiorespiratory Physiotherapy

Name airway clearance techniques suitable for a self-managing adult with cystic fibrosis.

Answer

Active cycle of breathing technique, autogenic drainage, oscillating positive expiratory pressure devices and PEP mask therapy. Choose with the patient for effectiveness, independence and adherence, as outcomes are generally comparable.

#388 · Cardiorespiratory Physiotherapy

What are the three components of the active cycle of breathing technique?

Answer

Breathing control, thoracic expansion exercises, and the forced expiration technique (huffing). Breathing control between cycles limits bronchospasm and fatigue, while huffs move secretions from the mid to upper airways.

#389 · Cardiorespiratory Physiotherapy

How are inhaled treatments usually sequenced around airway clearance in cystic fibrosis?

Answer

Typically bronchodilator first, then any mucoactive agent, then airway clearance, with inhaled antibiotics last so they reach cleared airways. Always follow the individual's medical plan and the CF team's advice.

#390 · Cardiorespiratory Physiotherapy

What cross infection precaution applies when treating several people with cystic fibrosis?

Answer

Avoid contact between people with CF in shared spaces, since organisms such as Pseudomonas aeruginosa and Burkholderia can transmit. Use separate sessions or rooms, clean equipment between uses and follow local infection control policy.

#391 · Cardiorespiratory Physiotherapy

What signs suggest a pulmonary exacerbation in a person with cystic fibrosis?

Answer

Increased cough and sputum, change in sputum colour or volume, more breathlessness, reduced exercise tolerance, weight loss and falling lung function. Report early to the CF team, as treatment often intensifies airway clearance.

#392 · Cardiorespiratory Physiotherapy

Why is regular exercise recommended for people with cystic fibrosis?

Answer

It supports airway clearance, aerobic fitness, muscle strength and bone health, and is linked with better quality of life. Tailor intensity to the person and treat it as a lifelong part of care.

#393 · Cardiorespiratory Physiotherapy

What should you monitor when a person with cystic fibrosis and reduced lung function exercises?

Answer

Oxygen saturation, heart rate, breathlessness and symptoms. Some people desaturate on exertion, so stop and rest if saturation falls below the threshold set by the team, and report the finding.

#394 · Cardiorespiratory Physiotherapy

Why do hydration and glucose matter when a person with cystic fibrosis exercises?

Answer

Sweat is salty, so salt and fluid loss rises in heat and long sessions. Those with CF related diabetes also need glucose monitoring. Advise fluids, snacks and a plan agreed with the CF team.

#395 · Cardiorespiratory Physiotherapy

A young adult with cystic fibrosis wants to stop treatments to fit in with peers. How do you respond?

Answer

Explore their reasons without judgement, share the benefits of treatment, set realistic shared goals and simplify the routine where possible. Respect their autonomy and involve the CF team for adherence support.

#396 · Cardiorespiratory Physiotherapy

What is the main goal of physiotherapy in advanced pulmonary fibrosis?

Answer

Symptom relief and quality of life rather than cure. Focus on breathlessness management, energy conservation, gentle activity and comfort in line with the person's goals, since the disease is progressive.

#397 · Cardiorespiratory Physiotherapy

Which non-drug strategies help breathlessness in end stage lung disease?

Answer

A handheld fan directed at the face, supportive positioning, pursed lip breathing, pacing and a calm environment. Cool airflow over the face can reduce the sensation of breathlessness.

#398 · Cardiorespiratory Physiotherapy

What is the role of oxygen in advanced pulmonary fibrosis?

Answer

It treats hypoxaemia and may ease breathlessness on exertion, but the medical team prescribes it. Not all breathlessness comes from low oxygen, so non-drug measures remain important alongside it.

#399 · Cardiorespiratory Physiotherapy

What does energy conservation teaching involve for a person with severe breathlessness?

Answer

Prioritising tasks, pacing with rests, sitting for activities such as showering, breathing out on effort, and using aids. This saves energy for the activities that matter most to the person.

#400 · Cardiorespiratory Physiotherapy

A person with pulmonary fibrosis wants to die at home. What is the physiotherapist's role?

Answer

Support their goals as part of the palliative care team, check that advance care planning is documented, and refer to community palliative services. Equipment, positioning and carer education help keep them comfortable.

#401 · Cardiorespiratory Physiotherapy

Does routine chest physiotherapy with percussion and vibration help an infant with uncomplicated bronchiolitis?

Answer

No. Evidence shows it does not shorten illness or hospital stay and can distress the infant. Care is supportive: oxygen, hydration, minimal handling and monitoring. Physiotherapy is reserved for specific complications such as lobar collapse.

#402 · Cardiorespiratory Physiotherapy

Which signs in a bronchiolitic infant suggest increasing respiratory distress and need urgent medical escalation?

Answer

Tachypnoea, nasal flaring, chest recession, grunting, apnoeic pauses, falling oxygen saturation, poor feeding and lethargy. Grunting, apnoea or lethargy signal fatigue and impending failure, so notify the medical team immediately.

#403 · Cardiorespiratory Physiotherapy

Why is gentle nasal suction sometimes useful before feeds in an infant with bronchiolitis?

Answer

Infants are obligate nose breathers, so secretions blocking the nares increase work of breathing and impair feeding. Suction should be gentle, brief and only when needed, because over-suctioning causes mucosal oedema and distress.

#404 · Cardiorespiratory Physiotherapy

What should you do when a parent of an infant with bronchiolitis asks you to explain the plan but speaks limited English?

Answer

Book a qualified interpreter (for example TIS National or hospital service) rather than using family members. This supports accurate informed consent, understanding of warning signs and safe discharge advice.

#405 · Cardiorespiratory Physiotherapy

What advice should be given to parents about reducing spread of bronchiolitis once the infant is home?

Answer

Hand hygiene, avoiding contact with other children, cleaning shared toys and surfaces, and keeping the infant away from cigarette smoke. Parents should return if feeding drops, breathing is laboured or the infant becomes drowsy.

#406 · Cardiorespiratory Physiotherapy

What is the rationale for prone positioning in a patient with moderate to severe ARDS?

Answer

Proning improves ventilation-perfusion matching, recruits dorsal lung regions and redistributes pressure, improving oxygenation. It is used for prolonged sessions under ICU protocol, and evidence supports a mortality benefit in severe ARDS.

#407 · Cardiorespiratory Physiotherapy

What lines and devices need securing and checking before and during turning a ventilated patient prone?

Answer

Endotracheal tube, central and arterial lines, chest drains, feeding tube, urinary catheter and ECG leads. Accidental extubation or line loss is the main risk, so a trained team with a designated airway lead is needed.

#408 · Cardiorespiratory Physiotherapy

What pressure injury risks arise during prone positioning and how are they reduced?

Answer

Face, chest, knees, iliac crests and eyes are at risk, plus facial oedema. Use padding, reposition the head and arms regularly, protect the eyes and check skin at each turn.

#409 · Cardiorespiratory Physiotherapy

When should proning be stopped or the patient returned supine in the ICU?

Answer

For cardiac arrest, severe haemodynamic instability, unplanned extubation or loss of airway, or major line displacement. Teams should know the emergency plan to return supine and start resuscitation beforehand.

#410 · Cardiorespiratory Physiotherapy

Why is a physiotherapist's scope in ARDS usually limited around mobilisation and secretion clearance?

Answer

Severe hypoxaemia, high ventilator settings and sedation or paralysis limit what is safe. Decisions follow ICU team agreement and set safety thresholds. Physiotherapy focuses on positioning, limited secretion clearance and early rehabilitation as stability returns.

#411 · Cardiorespiratory Physiotherapy

What is intensive care acquired weakness and how does it typically present?

Answer

Symmetrical, generalised limb and respiratory muscle weakness arising during critical illness, often with preserved facial muscles and reduced reflexes. It prolongs ventilation and hospital stay and is a key reason for early rehabilitation.

#412 · Cardiorespiratory Physiotherapy

Which Medical Research Council scale assessment is used to identify ICU acquired weakness and what score suggests it?

Answer

The MRC sum score tests six muscle groups bilaterally, graded 0 to 5, total 60. A total below 48 indicates clinically significant weakness. It needs a cooperative, alert patient who can follow commands.

#413 · Cardiorespiratory Physiotherapy

What are the main risk factors for developing ICU acquired weakness?

Answer

Prolonged mechanical ventilation, sepsis, multi-organ failure, immobility, hyperglycaemia, corticosteroids and neuromuscular blockers. Minimising sedation, controlling glucose and mobilising early are the modifiable strategies.

#414 · Cardiorespiratory Physiotherapy

What are common safety criteria to start early mobilisation of a ventilated ICU patient?

Answer

Stable haemodynamics without escalating vasopressors, acceptable oxygenation and ventilator settings, adequate arousal and no uncontrolled arrhythmia or raised intracranial pressure. Each unit has local criteria that should be followed.

#415 · Cardiorespiratory Physiotherapy

Which outcome measure suits tracking functional progress in an ICU patient who is awake and following commands?

Answer

The Physical Function in ICU Test or the ICU Mobility Scale. They measure function and level of mobility achieved, showing progression from bed activity to walking and guiding goal setting and discharge planning.

#416 · Cardiorespiratory Physiotherapy

What are the typical clinical features of an acute myocardial infarction that warrant immediate medical attention?

Answer

Central chest pain or pressure possibly radiating to arm, jaw or back, with sweating, nausea and breathlessness. Presentation can be atypical in women, older people and people with diabetes. Stop treatment and call for help.

#417 · Cardiorespiratory Physiotherapy

When can early cardiac rehabilitation begin after an uncomplicated myocardial infarction?

Answer

Low-level mobilisation can begin in hospital once the patient is stable, pain-free and without arrhythmia or heart failure signs. Structured outpatient programs follow and reduce readmission and mortality, so referral before discharge is standard.

#418 · Cardiorespiratory Physiotherapy

What intensity guides are used to prescribe safe exercise after a myocardial infarction?

Answer

Rating of perceived exertion, heart rate limits set by the cardiologist or exercise test, and symptom response. Beta-blockers blunt heart rate, so perceived exertion becomes the more reliable guide for intensity.

#419 · Cardiorespiratory Physiotherapy

Which findings during exercise after a myocardial infarction require stopping the session?

Answer

Chest pain, marked breathlessness, dizziness, pallor, a fall in blood pressure with increasing workload, new arrhythmia or abnormal heart rate response. Stop, position the patient safely, monitor and report to the medical team.

#420 · Cardiorespiratory Physiotherapy

What secondary prevention education should be covered in cardiac rehabilitation after a myocardial infarction?

Answer

Medication adherence, smoking cessation, physical activity, diet, blood pressure and cholesterol control, stress and mood, and symptom action plans. Depression is common, so screen and refer when indicated.

#421 · Cardiorespiratory Physiotherapy

What are sternal precautions after coronary artery bypass grafting through a median sternotomy?

Answer

Protect the healing sternum by avoiding heavy lifting, pushing and pulling with the arms, and unsupported coughing. Typical guidance lasts about six to twelve weeks, but follow the surgeon's local protocol.

#422 · Cardiorespiratory Physiotherapy

How should a patient be taught to cough and huff after sternotomy?

Answer

Hug a firm pillow or folded towel against the chest while coughing or huffing to splint the sternum. This reduces pain and sternal stress while allowing effective secretion clearance and lowering the risk of chest infection.

#423 · Cardiorespiratory Physiotherapy

What signs of sternal wound complication should prompt urgent referral after CABG?

Answer

Sternal clicking or instability, wound redness, discharge, increasing pain, fever or a gaping wound. These may indicate infection or dehiscence, so notify the surgical team promptly and avoid loading the arms.

#424 · Cardiorespiratory Physiotherapy

Why is a graft harvest site in the leg also assessed after CABG?

Answer

Saphenous vein harvest can cause leg swelling, wound infection and pain. Assess the wound, oedema and calf signs of deep vein thrombosis. Encourage walking and elevation, and report infection or wound breakdown.

#425 · Cardiorespiratory Physiotherapy

What is the role of delegation to an allied health assistant in post-CABG mobilisation?

Answer

A stable patient on an established walking program may be supervised by an allied health assistant under a physiotherapist's written plan. The physiotherapist remains accountable, and any change in condition is escalated back.

#426 · Cardiorespiratory Physiotherapy

What does heart failure with reduced ejection fraction mean, and why does it limit exercise tolerance?

Answer

The left ventricle pumps out a reduced share of its blood each beat, so cardiac output cannot rise normally with effort. Patients fatigue and become breathless early, so start low and progress gradually.

#427 · Cardiorespiratory Physiotherapy

A patient with heart failure takes a beta-blocker. Why is a heart rate target unreliable for prescribing exercise intensity?

Answer

Beta-blockers blunt the heart rate response to effort, so heart rate underestimates workload. Use the Borg RPE scale and the talk test instead, and monitor symptoms and blood pressure.

#428 · Cardiorespiratory Physiotherapy

During a walk, a patient with heart failure has a fall in systolic blood pressure with increasing workload. What should you do?

Answer

Stop the exercise, sit or lie the patient down and recheck observations. A falling blood pressure with effort suggests the heart cannot meet demand. Report to the medical team before further exercise.

#429 · Cardiorespiratory Physiotherapy

What exercise approach suits a stable patient with heart failure with reduced ejection fraction in a rehabilitation programme?

Answer

Supervised aerobic exercise at a low to moderate intensity, progressed slowly, with warm-up and cool-down and light resistance work. Regular training improves function and symptoms, but only once the patient is clinically stable.

#430 · Cardiorespiratory Physiotherapy

How can a patient with chronic heart failure access Medicare-subsidised physiotherapy and cardiac rehabilitation in Australia?

Answer

The GP can refer the patient to a cardiac rehabilitation program (often hospital or state-funded rather than a Medicare item). Separately, a GP Management Plan with Team Care Arrangements allows a limited number of Medicare-subsidised allied health visits each year (currently up to 5). Check the plan and referral before starting treatment.

#431 · Cardiorespiratory Physiotherapy

Why do you teach a patient with heart failure to weigh themselves each morning?

Answer

A rapid weight gain over a few days usually means fluid retention, not fat gain. Early detection lets the GP or heart failure nurse adjust treatment before the patient needs hospital admission.

#432 · Cardiorespiratory Physiotherapy

What does pitting oedema in the ankles and lower legs suggest in a patient with known heart failure?

Answer

Fluid retention from raised venous pressure, often from right-sided heart failure. Record the extent, compare sides and over time, and report any increase to the medical team rather than treating it as a local leg problem.

#433 · Cardiorespiratory Physiotherapy

A patient with heart failure cannot lie flat and wakes at night short of breath. What are these symptoms called and how do you position them?

Answer

Orthopnoea and paroxysmal nocturnal dyspnoea, both signs of pulmonary congestion. Use high side-lying or upright supported sitting to ease breathing, and report new or worsening symptoms to the treating team.

#434 · Cardiorespiratory Physiotherapy

A heart failure patient has new breathlessness at rest and new crackles on auscultation. Should you start the planned exercise session?

Answer

No. These findings suggest acute decompensation with pulmonary oedema. Sit the patient upright, check observations, withhold exercise and escalate urgently to the medical team according to local deterioration protocols.

#435 · Cardiorespiratory Physiotherapy

A physiotherapist delegates a stable heart failure patient's walking programme to an allied health assistant. What must the physiotherapist retain?

Answer

Responsibility for assessment, programme design, stop criteria and reassessment. The assistant follows clear written instructions and reports symptoms or abnormal observations immediately. Delegation requires training, supervision and a patient who is stable and predictable.

#436 · Cardiorespiratory Physiotherapy

What are sternal precautions after valve surgery through a median sternotomy, and why are they taught?

Answer

Avoid pushing, pulling and lifting heavy loads with the arms, and avoid unsupported reaching behind. They protect healing of the divided sternum and reduce the risk of instability or wound breakdown.

#437 · Cardiorespiratory Physiotherapy

How do you teach a patient to cough safely after a sternotomy for valve replacement?

Answer

Hug a firm pillow or rolled towel firmly against the chest wall while coughing. Splinting reduces sternal movement and pain, so the patient can clear secretions and avoid post-operative chest complications.

#438 · Cardiorespiratory Physiotherapy

After transcatheter aortic valve implantation through the femoral artery, what should you check at the access site during early mobilisation?

Answer

Look at the groin for bleeding, swelling, haematoma or new pain, and check distal pulses and limb colour. Report any change promptly, as vascular access complications can develop soon after mobilising.

#439 · Cardiorespiratory Physiotherapy

A patient with a new mechanical heart valve is on warfarin. What precaution applies to your physiotherapy?

Answer

Bleeding and bruising risk is raised, so avoid vigorous soft tissue techniques, take care with falls and handling, and report unexplained bruising or bleeding. Anticoagulation is essential to prevent valve thrombosis.

#440 · Cardiorespiratory Physiotherapy

A patient recovering from valve surgery has limited English. How should you obtain consent and teach precautions?

Answer

Book a qualified interpreter, ideally an accredited one, rather than relying on family members. Use teach-back to confirm understanding of sternal precautions and warning signs, and document that the interpreter was used.

#441 · Cardiorespiratory Physiotherapy

Why do patients with a known abdominal aortic aneurysm avoid breath-holding and straining during exercise?

Answer

The Valsalva manoeuvre sharply raises blood pressure and wall stress on the aneurysm. Teach the patient to breathe out through effort and avoid heavy isometric or maximal lifts.

#442 · Cardiorespiratory Physiotherapy

A patient with a known aortic aneurysm reports sudden severe tearing chest or back pain during activity. What is your response?

Answer

Treat it as a possible rupture or dissection: stop activity, lie the patient down, monitor observations and call an ambulance immediately. Do not continue assessment, and keep the patient still until emergency help arrives.

#443 · Cardiorespiratory Physiotherapy

Before starting exercise with a patient who has an aortic aneurysm, what must you clarify with the treating medical team?

Answer

The maximum safe blood pressure and any activity limits for that patient's aneurysm size and plan. Record these limits in the notes and monitor blood pressure against them before, during and after exercise.

#444 · Cardiorespiratory Physiotherapy

Which type of exercise is generally preferred for a patient with an aortic aneurysm under surveillance?

Answer

Low to moderate aerobic exercise such as walking at a steady pace, with blood pressure kept within the limits set by the medical team. Avoid heavy resistance and maximal isometric efforts that spike blood pressure.

#445 · Cardiorespiratory Physiotherapy

Why must the blood pressure limits for a patient with an aortic aneurysm be documented in the record?

Answer

So every clinician treating the patient works to the same limits, including any assistant you delegate to. Clear documentation supports safe practice, handover and a defensible record if the patient deteriorates.

#446 · Cardiorespiratory Physiotherapy

While taking the pulse before exercise you find it irregular and the patient is dizzy. What is the correct action?

Answer

Do not start exercise. Check blood pressure, oxygen saturation and symptoms, and escalate to the medical team for an ECG and review. A new irregular rhythm with symptoms may indicate unstable atrial fibrillation or another arrhythmia.

#447 · Cardiorespiratory Physiotherapy

Name the criteria for stopping a cardiac exercise session based on symptoms or observations.

Answer

Chest pain, marked breathlessness, dizziness or faintness, pallor, a fall in systolic blood pressure, a new arrhythmia or ST changes on monitoring. Stop, rest the patient and assess before deciding on next steps.

#448 · Cardiorespiratory Physiotherapy

A patient has an implantable cardioverter defibrillator. How should you set exercise intensity relative to the device?

Answer

Keep the exercise heart rate safely below the lowest programmed ICD detection rate (commonly at least 10 to 20 bpm below). Obtain that rate from the cardiology or device clinic in writing, and stop exercise if the patient feels palpitations or lightheadedness.

#449 · Cardiorespiratory Physiotherapy

What shoulder and arm advice applies in the first weeks after a new pacemaker implantation?

Answer

Follow the surgeon's written instructions, which usually limit vigorous arm and shoulder movement on the implant side while the leads settle. Avoid heavy lifting or overhead work until cleared, and report swelling or wound redness.

#450 · Cardiorespiratory Physiotherapy

A patient faints during a supervised exercise session and recovers. After immediate care, what are your professional obligations?

Answer

Report the event through the facility's incident reporting system, notify the treating doctor and document observations, actions and the patient's response. Do not resume the programme until medical review has cleared it.

#451 · Cardiorespiratory Physiotherapy

A patient with a diabetic foot ulcer has cool, pale feet and absent pedal pulses. What does this suggest and what should you do first?

Answer

It suggests peripheral arterial disease limiting perfusion, which impairs healing. Check pulses and capillary refill, avoid compression or vigorous debridement-type loading, and refer to the GP or vascular team before progressing treatment.

#452 · Cardiorespiratory Physiotherapy

Why is offloading the central priority in managing a plantar diabetic foot ulcer?

Answer

Repetitive pressure on the ulcer site prevents healing and drives deterioration. Use offloading footwear or devices as prescribed by the podiatry or high-risk foot team, and educate the patient to reduce weight-bearing on the site.

#453 · Cardiorespiratory Physiotherapy

A patient with diabetic neuropathy wants to start a walking programme. What foot safety advice should you give?

Answer

Check both feet daily, wear well-fitting protective footwear and socks, never walk barefoot, and stop and report any blister, redness or break in skin. Loss of protective sensation means damage goes unnoticed.

#454 · Cardiorespiratory Physiotherapy

Which symptom pattern of intermittent claudication guides exercise prescription in peripheral arterial disease?

Answer

Calf pain on walking that settles with rest. Prescribe repeated walking to moderate claudication pain, rest until it eases, then repeat. This interval approach improves walking tolerance, but rest pain or tissue loss needs medical review first.

#455 · Cardiorespiratory Physiotherapy

A patient with a foot ulcer asks you to apply the dressing in clinic, but you are not trained in wound care. What is the correct response?

Answer

Work within your scope and competence: refer to a wound care nurse, podiatrist or the multidisciplinary high-risk foot service. Practising outside your competence risks patient harm and breaches your professional obligations to AHPRA and the Board.

#456 · Cardiorespiratory Physiotherapy

Which calf findings raise suspicion of deep vein thrombosis during a post-operative assessment?

Answer

Unilateral calf swelling, warmth, tenderness and redness, especially after surgery or immobility. Do not massage or vigorously mobilise the limb. Withhold treatment and urgently refer for medical assessment and imaging.

#457 · Cardiorespiratory Physiotherapy

A post-operative patient suddenly develops pleuritic chest pain, breathlessness and tachycardia. What is the physiotherapist's response?

Answer

Suspect pulmonary embolism, a medical emergency. Stop treatment, keep the patient supported and calm, monitor vital signs, and call for urgent medical help or emergency services in line with local escalation procedures.

#458 · Cardiorespiratory Physiotherapy

What are common risk factors for venous thromboembolism that you should screen for before mobilising a patient?

Answer

Recent surgery, prolonged immobility, previous clot, malignancy, pregnancy, obesity and the oral contraceptive pill. Identifying risk allows early escalation and supports safe, graded mobilisation rather than delayed recognition.

#459 · Cardiorespiratory Physiotherapy

Why is early mobilisation encouraged for patients at risk of DVT once medically cleared?

Answer

Walking activates the calf muscle pump and promotes venous return, reducing stasis. Mobilise only once a confirmed DVT has been ruled out or treated, following medical advice on anticoagulation and activity limits.

#460 · Cardiorespiratory Physiotherapy

You suspect a DVT in a patient but the ward doctor is busy. How should you document and escalate?

Answer

Record the findings objectively, stop treatment, and escalate directly to the treating medical team using the local escalation pathway. Document who you told, the time and the response, so the care trail is clear.

#461 · Cardiorespiratory Physiotherapy

What does obesity hypoventilation syndrome involve, and how does it differ from simple obesity?

Answer

It involves daytime hypercapnia in an obese patient, driven by reduced respiratory drive and mechanics, often alongside sleep disordered breathing. Simple obesity alone does not cause chronic daytime carbon dioxide retention.

#462 · Cardiorespiratory Physiotherapy

Why must supplemental oxygen be titrated carefully in a patient with obesity hypoventilation?

Answer

High-flow oxygen can suppress respiratory drive and worsen carbon dioxide retention. Use a prescribed target saturation range, monitor for drowsiness or confusion, and escalate promptly if the patient becomes sleepy or hypercapnic.

#463 · Cardiorespiratory Physiotherapy

Which positioning strategy helps ventilation in an obese patient with breathing difficulty?

Answer

Upright or high side-lying positions reduce abdominal load on the diaphragm and improve lung volumes. Lying flat worsens breathlessness, so use sitting or reclined positions during rest and treatment.

#464 · Cardiorespiratory Physiotherapy

A patient reports loud snoring, witnessed apnoeas and daytime sleepiness. What does this suggest?

Answer

It suggests obstructive sleep apnoea. Refer to the GP for a sleep study and review rather than diagnosing it yourself. Untreated sleep apnoea raises cardiovascular risk and affects safety, for example when driving.

#465 · Cardiorespiratory Physiotherapy

What is the role of exercise in a patient with obesity and sleep disordered breathing?

Answer

Regular graded aerobic and strengthening exercise supports weight management, fitness and breathlessness tolerance. Monitor saturation and symptoms, start at an achievable level, and tailor progression to the patient's comorbidities and goals.

#466 · Cardiorespiratory Physiotherapy

A patient with motor neurone disease has a weak, ineffective cough. Why is this clinically important?

Answer

Weak expiratory muscles cannot clear secretions, raising the risk of retained mucus, atelectasis and chest infection. Assess cough strength and plan airway clearance support early rather than waiting for a respiratory complication.

#467 · Cardiorespiratory Physiotherapy

What does a mechanical insufflation-exsufflation (cough assist) device do?

Answer

It delivers a positive pressure breath, then rapidly switches to negative pressure to simulate a cough and draw secretions out. It suits patients whose own cough is too weak to clear their airway.

#468 · Cardiorespiratory Physiotherapy

Why is the peak cough flow a useful measure in neuromuscular disease?

Answer

It indicates how effectively the patient can clear secretions. A low value signals the need for assisted cough techniques, helping you decide when to introduce manual or mechanical cough assistance.

#469 · Cardiorespiratory Physiotherapy

How can a manually assisted cough be performed for a patient who cannot generate an effective cough?

Answer

After a maximal inspiration, apply a firm, timed abdominal or lower chest thrust during the expiratory phase. Time it to the cough effort, avoid it straight after meals, and check for contraindications such as recent abdominal surgery.

#470 · Cardiorespiratory Physiotherapy

A patient with progressive neuromuscular disease declines non-invasive ventilation. How do you respond?

Answer

Respect their autonomy once they have capacity and informed information. Explore their reasons, ensure they understand the benefits and risks, document the discussion, and involve the medical and palliative teams in ongoing care planning.

#471 · Cardiorespiratory Physiotherapy

Why is chest physiotherapy in infants often delivered in positions different from adult techniques?

Answer

Infants have smaller, more compliant airways and a different chest wall, so gentle positioning and age-appropriate techniques are used. Always use paediatric-specific methods rather than adult techniques, which may be unsafe.

#472 · Cardiorespiratory Physiotherapy

What must you do before starting chest physiotherapy on a child?

Answer

Obtain informed consent from the parent or guardian, explain the technique in age-appropriate terms, and check the indication with the medical team. Treatment is not routine, and unnecessary physiotherapy in a stable child adds distress.

#473 · Cardiorespiratory Physiotherapy

What is a sign of respiratory distress in a young child that should prompt you to stop and escalate?

Answer

Increased work of breathing such as grunting, nasal flaring, chest wall recession, a fast respiratory rate, or lethargy. Stop treatment, support the child, and call the medical team urgently.

#474 · Cardiorespiratory Physiotherapy

How should an interpreter be used when treating a child whose family has limited English?

Answer

Book a qualified interpreter rather than relying on family members, so consent and instructions are accurate. Speak to the parent, not the interpreter, and allow extra time to confirm understanding of the treatment plan.

#475 · Cardiorespiratory Physiotherapy

Why is the Borg rating of perceived exertion useful when prescribing exercise for a person taking a beta blocker?

Answer

Beta blockers blunt the heart rate response, so heart rate targets can mislead. The person's own effort rating, with symptoms and blood pressure, gives a more reliable guide to intensity, commonly aiming for somewhat hard.

#476 · Cardiorespiratory Physiotherapy

A patient with lung cancer reports new bone pain and a recent minor fall. What should the physiotherapist consider before exercise?

Answer

Possible bone metastases and fracture risk. Pause loading and resisted exercise, limit twisting and impact, and refer urgently to the medical team for imaging before progressing. Pathological fracture can follow trivial load.

#477 · Cardiorespiratory Physiotherapy

Why is preoperative physiotherapy useful before lung resection for cancer?

Answer

Education, breathing exercise training and exercise conditioning improve fitness and can reduce postoperative pulmonary complications. The patient also learns coughing with wound support and early mobilisation before surgery, so they can do it when sore.

#478 · Cardiorespiratory Physiotherapy

After a thoracotomy, how should a patient be taught to cough and why?

Answer

Teach supported huffing and coughing, hugging a rolled towel or pillow over the wound, with adequate analgesia timed before treatment. Pain limits deep breaths and cough, raising the risk of sputum retention and atelectasis.

#479 · Cardiorespiratory Physiotherapy

A person with advanced lung cancer has significant weight loss and muscle wasting. What does this suggest, and how does it affect exercise prescription?

Answer

Cancer cachexia, with loss of muscle and function. Use low-intensity, individualised exercise with rest breaks, monitor fatigue and symptoms, and work with the dietitian. Overly vigorous exercise can worsen fatigue and energy deficit.

#480 · Cardiorespiratory Physiotherapy

A patient with lung cancer from a remote community lives hours from the treating centre. How can physiotherapy continue?

Answer

Use telehealth reviews, a written home exercise program and liaison with local health services or the Aboriginal health service. Plan with the patient and family, and use an interpreter where needed, so care continues close to home.

#481 · Cardiorespiratory Physiotherapy

What are the typical features of post COVID-19 breathlessness and fatigue that guide treatment pacing?

Answer

Symptoms may fluctuate and worsen after exertion, sometimes a day or two later (post-exertional symptom exacerbation). Pace activity, avoid pushing through symptoms, and progress very gradually using symptom response rather than a fixed schedule.

#482 · Cardiorespiratory Physiotherapy

A post COVID-19 patient has new chest pain, palpitations or a resting heart rate spike on mild exertion. What is your response?

Answer

Stop exercise, check vital signs and refer to the GP or medical team for review before resuming. These can signal cardiac complications such as myocarditis or arrhythmia, which require medical clearance before exertion.

#483 · Cardiorespiratory Physiotherapy

Why use a symptom diary and pacing plan in post COVID-19 fatigue?

Answer

A diary links activity with symptom flares, letting the patient and physiotherapist set sustainable activity levels. It teaches energy conservation, prevents boom and bust cycles and guides when it is safe to increase activity.

#484 · Cardiorespiratory Physiotherapy

What breathing strategies suit a post COVID-19 patient with dysfunctional breathing and breathlessness?

Answer

Diaphragmatic and relaxed lower-chest breathing, controlled pacing of breaths and positions that ease breathlessness such as forward lean. Reduce upper chest effort and anxiety. Rule out medical causes first and explain why this helps.

#485 · Cardiorespiratory Physiotherapy

A patient with long COVID asks to return to work and the gym immediately. How do you handle this?

Answer

Explain graded return with the patient setting goals, use a workplace plan with employer consent, and review symptoms after each step. Share information only with the patient's consent. Rushed return risks relapse and prolonged fatigue.

#486 · Cardiorespiratory Physiotherapy

A pregnant woman with asthma reports increased wheeze and breathlessness at rest. What is the key physiotherapy action?

Answer

Treat as possibly an acute exacerbation: check vital signs and oxygen saturation, support her to use her reliever per her asthma action plan and seek urgent medical review. Poorly controlled asthma risks both mother and fetal oxygenation.

#487 · Cardiorespiratory Physiotherapy

Why does breathlessness commonly increase in later pregnancy, and how does this affect interpreting cardiorespiratory signs?

Answer

The growing uterus raises the diaphragm and progesterone increases ventilatory drive, so mild breathlessness is common. Breathlessness at rest, chest pain or marked exercise intolerance is not normal and needs medical assessment.

#488 · Cardiorespiratory Physiotherapy

What positioning precaution applies when treating a woman in later pregnancy with a cardiac condition?

Answer

Avoid prolonged lying flat on her back, where the uterus can compress the inferior vena cava and reduce venous return. Use side-lying or a wedge or semi-reclined position, and watch for dizziness or hypotension.

#489 · Cardiorespiratory Physiotherapy

How should exercise intensity be monitored in a pregnant woman with known cardiac disease?

Answer

Use symptoms, perceived exertion and talk test rather than relying on target heart rate, which is less reliable in pregnancy. Stop and seek review for chest pain, dizziness, palpitations or breathlessness out of proportion to effort.

#490 · Cardiorespiratory Physiotherapy

A pregnant patient with cystic fibrosis wants to continue airway clearance. What should the physiotherapist consider?

Answer

Continue airway clearance with the specialist team, adapting positions to avoid head-down tilt and pressure on the abdomen, and favouring techniques she can use comfortably. Coordinate with the obstetric and CF teams and monitor lung function and nutrition.

#491 · Cardiorespiratory Physiotherapy

What standard precautions must a physiotherapist apply for every patient in respiratory care?

Answer

Hand hygiene at the five moments, appropriate personal protective equipment based on risk of exposure to secretions, safe handling of sharps and equipment, and cleaning of shared devices. Standard precautions apply regardless of known infection status.

#492 · Cardiorespiratory Physiotherapy

Why are cough-inducing or aerosol-generating respiratory treatments a particular infection risk?

Answer

Coughing, suction, nebulisation and some airway clearance techniques release droplets and aerosols. Use transmission-based precautions with appropriate PPE such as a fit-checked P2 or N95 respirator, and limit these treatments to cases where there is clear clinical benefit.

#493 · Cardiorespiratory Physiotherapy

A patient in airborne isolation needs respiratory physiotherapy. Which precautions and room arrangements apply?

Answer

Use a negative pressure or single room with the door closed, a fit-checked P2 or N95 respirator, eye protection, gown and gloves, and minimise staff entering. Follow the facility's infection control policy and seek advice from the infection control team.

#494 · Cardiorespiratory Physiotherapy

How should shared respiratory equipment such as spirometers or mouthpieces be managed between patients?

Answer

Use single-patient items where possible, such as individual mouthpieces and filters, and clean or disinfect reusable equipment after each patient per policy. Shared equipment can transmit infection between patients.

#495 · Cardiorespiratory Physiotherapy

A physiotherapist develops a cough and fever after treating patients on a respiratory ward. What is the professional responsibility?

Answer

Do not attend work while symptomatic, follow workplace health and infection control policy, notify the manager and the infection control or occupational health team, and report as required. Protecting patients and colleagues takes priority over staffing pressure.

#496 · Cardiorespiratory Physiotherapy

What does the six-minute walk test measure and how is it used in cardiorespiratory physiotherapy?

Answer

The distance walked in six minutes on a flat course, a measure of functional exercise capacity. It tracks response to rehabilitation or disease progression, and is recorded with symptoms, heart rate and oxygen saturation to guide exercise prescription.

#497 · Cardiorespiratory Physiotherapy

Why is the Borg scale used with exercise testing in a patient with breathlessness?

Answer

It records perceived breathlessness or exertion on a numeric scale. It gives a subjective intensity target that guides prescription, shows change over time and helps identify when to stop or modify exercise.

#498 · Cardiorespiratory Physiotherapy

When must a six-minute walk test or exercise test be stopped?

Answer

Stop for chest pain, severe breathlessness, dizziness, pallor, leg cramps, a marked fall in oxygen saturation or any intolerable symptom, and for staff concern. Follow the test protocol and report abnormal responses to the medical team.

#499 · Cardiorespiratory Physiotherapy

Why should resting vital signs and contraindications be checked before an exercise test in a person with cardiorespiratory disease?

Answer

Unstable angina, uncontrolled arrhythmia, a recent cardiac event, severe resting hypertension or hypoxia can make exertion unsafe. Screening identifies who needs medical clearance first and sets baseline values to compare during and after the test.

#500 · Cardiorespiratory Physiotherapy

A patient's six-minute walk distance has improved after pulmonary rehabilitation. How should you interpret and communicate this?

Answer

Compare with the patient's baseline and recognised minimal important difference for their condition, and consider symptoms and function too. Share the result in plain language, link it to their goals and use it to guide maintenance.